

MRN: 35294426

Date of Visit: May 23, 2025

Oncologist: Dr. Michael Chen, MD

Chief Complaint

- Early satiety, Abdominal pain, Constipation

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 23, 2025
- Site of primary tumor: Descending colon (left colon)
- Date of initial diagnosis and TNM stage: April 23, 2025; T2N0M1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 06, 2025 - liver, brain metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Early satiety, Abdominal pain, Constipation
- Performance status: ECOG 0
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Hypertension
 - Chronic back pain
 - Hypertension
 - COPD
- Surgical history:
 - Joint replacement (2009)

Family History

- Colorectal or other cancers in first-degree relatives: Paternal uncle with prostate cancer
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Current smoker (10 pack-years)
- Alcohol use: Rare alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired teacher
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency

- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Metoprolol 50mg twice daily
- Multivitamin daily
- Allergies: Penicillin (rash), Shellfish (hives)

Physical Examination

- Vitals: BP 115/77, HR 83, RR 20, Temp 98.8°F, SpO2 97% on room air, Weight 157 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $9.0 \times 10^3/\mu\text{L}$, ANC: $4.9 \times 10^3/\mu\text{L}$
- Hgb: 8.5 g/dL, Platelets: $190 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.0 mg/dL, eGFR: 100 mL/min/1.73m²
- LFTs:
- AST: 34 U/L, ALT: 29 U/L
- Alkaline phosphatase: 224 U/L, Total bilirubin: 2.5 mg/dL
- CEA: 59.6 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 06, 2025):
- Primary descending colon mass (6.9 cm), circumferential thickening
- 3 hepatic metastases (largest 6.0 cm), 2 brain metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (liver, brain)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 59779266

Date of Visit: June 17, 2025

Oncologist: Dr. Lisa Park, MD

Chief Complaint

- Cramping, Nausea, Fatigue

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 16, 2025
- Site of primary tumor: Sigmoid colon (left colon)
- Date of initial diagnosis and TNM stage: May 16, 2025; T3N2aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 09, 2025 - lymph nodes metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Cramping, Nausea, Fatigue, Diarrhea
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Osteoporosis
 - Dementia (early stage)
 - Hypertension
 - Osteoarthritis
- Surgical history:
 - Appendectomy (2012)
 - Tonsillectomy (1991)

Family History

- Colorectal or other cancers in first-degree relatives: Mother with breast cancer at age 72
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Current smoker (10 pack-years)
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Sales manager
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing

- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Metoprolol 50mg twice daily
- Multivitamin daily
- Allergies: Penicillin (rash), Shellfish (hives)

Physical Examination

- Vitals: BP 131/78, HR 81, RR 14, Temp 98.7°F, SpO2 97% on room air, Weight 124 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $4.1 \times 10^3/\mu\text{L}$, ANC: $1.9 \times 10^3/\mu\text{L}$
- Hgb: 13.3 g/dL, Platelets: $351 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.0 mg/dL, eGFR: 96 mL/min/1.73m²
- LFTs:
- AST: 27 U/L, ALT: 14 U/L
- Alkaline phosphatase: 54 U/L, Total bilirubin: 0.5 mg/dL
- CEA: 13.9 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSI-H

- Imaging:

- CT chest/abdomen/pelvis (June 09, 2025):
- Primary sigmoid colon mass (3.7 cm), annular narrowing
- enlarged lymph nodes (largest 3.0 cm)
- Other findings: Hiatal hernia
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lymph nodes)
- Disease status: Newly diagnosed, treatment-naïve

- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 30913714

Date of Visit: May 23, 2025

Oncologist: Dr. David Thompson, MD

Chief Complaint

- Early satiety, Abdominal pain, Weight loss

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 26, 2025
- Site of primary tumor: Upper rectum (rectum)
- Date of initial diagnosis and TNM stage: April 26, 2025; T2N2bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 03, 2025 - brain, lymph nodes metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Early satiety, Abdominal pain, Weight loss
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Chronic kidney disease
 - Psoriasis
 - COPD
 - Dementia (early stage)
 - Chronic back pain
 - Stroke history
- Surgical history:
 - Testicular surgery (1999)
 - Circumcision (2012)

Family History

- Colorectal or other cancers in first-degree relatives: No known family history of cancer
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Former smoker, quit 5 years ago (15 pack-year history)
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Engineer
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety

- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Multivitamin daily
- Allergies: Penicillin (rash), Shellfish (hives)

Physical Examination

- Vitals: BP 115/79, HR 83, RR 14, Temp 98.0°F, SpO2 96% on room air, Weight 191 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $12.9 \times 10^3/\mu\text{L}$, ANC: $6.7 \times 10^3/\mu\text{L}$
- Hgb: 12.0 g/dL, Platelets: $302 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.3 mg/dL, eGFR: 73 mL/min/1.73m²
- LFTs:
- AST: 94 U/L, ALT: 70 U/L
- Alkaline phosphatase: 125 U/L, Total bilirubin: 1.4 mg/dL
- CEA: 104.6 ng/mL (elevated)
- Molecular markers:
- RAS: NRAS Q61H
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 03, 2025):
- Primary upper rectum mass (6.7 cm), polypoid mass
- enlarged lymph nodes (largest 3.4 cm), 4 brain metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (brain, lymph nodes)
- Disease status: Newly diagnosed, treatment-naïve

- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 98938054

Date of Visit: June 17, 2025

Oncologist: Dr. Sarah Mitchell, MD

Chief Complaint

- Nausea, Cramping, Weight loss

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 31, 2025
- Site of primary tumor: Transverse colon (right colon)
- Date of initial diagnosis and TNM stage: May 31, 2025; T4bN2aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 16, 2025 - peritoneum, lung metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Nausea, Cramping, Weight loss, Constipation
- Performance status: ECOG 0
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Celiac disease
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Father with colon cancer at age 68
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Never smoker
- Alcohol use: Rare alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Office administrator
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes

- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Sulfa drugs (GI upset)

Physical Examination

- Vitals: BP 112/86, HR 96, RR 16, Temp 99.2°F, SpO2 96% on room air, Weight 201 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $6.8 \times 10^3/\mu\text{L}$, ANC: $8.8 \times 10^3/\mu\text{L}$
- Hgb: 14.3 g/dL, Platelets: $375 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.9 mg/dL, eGFR: 31 mL/min/1.73m²
- LFTs:
- AST: 46 U/L, ALT: 14 U/L
- Alkaline phosphatase: 64 U/L, Total bilirubin: 2.9 mg/dL
- CEA: 101.7 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: V600E
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 16, 2025):
- Primary transverse colon mass (6.6 cm), circumferential thickening
- 6 pulmonary nodules (largest 4.0 cm), peritoneal disease
- Other findings: Degenerative spine changes
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (peritoneum, lung)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 21080851

Date of Visit: June 14, 2025

Oncologist: Dr. Sarah Mitchell, MD

Chief Complaint

- Rectal bleeding, Vomiting, Abdominal pain

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 21, 2025
- Site of primary tumor: Transverse colon (right colon)
- Date of initial diagnosis and TNM stage: May 21, 2025; T4bN1aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 14, 2025 - lymph nodes, bone, brain metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Rectal bleeding, Vomiting, Abdominal pain, Bloating, Constipation
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Hyperlipidemia
 - GERD
 - Fibromyalgia
 - Type 2 diabetes
 - Heart failure
- Surgical history:
 - Skin lesion removal (2018)
 - Arthroscopy (1996)

Family History

- Colorectal or other cancers in first-degree relatives: No known family history of cancer
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Former smoker, quit 10 years ago (20 pack-year history)
- Alcohol use: Rare alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Nurse
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath

- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Metformin 1000mg twice daily
- Atorvastatin 20mg daily
- Multivitamin daily
- Allergies: Penicillin (rash)

Physical Examination

- Vitals: BP 138/85, HR 71, RR 20, Temp 99.0°F, SpO2 100% on room air, Weight 178 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $9.3 \times 10^3/\mu\text{L}$, ANC: $4.3 \times 10^3/\mu\text{L}$
- Hgb: 13.2 g/dL, Platelets: $433 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.9 mg/dL, eGFR: 93 mL/min/1.73m²
- LFTs:
- AST: 28 U/L, ALT: 17 U/L
- Alkaline phosphatase: 124 U/L, Total bilirubin: 0.5 mg/dL
- CEA: 101.5 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: V600E
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 14, 2025):
- Primary transverse colon mass (2.8 cm), infiltrative mass
- enlarged lymph nodes (largest 2.7 cm), osseous metastases, 1 brain metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lymph nodes, bone, brain)

- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 29936192

Date of Visit: June 08, 2025

Oncologist: Dr. David Thompson, MD

Chief Complaint

- Weight loss, Fatigue, Early satiety

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 15, 2025
- Site of primary tumor: Ascending colon (right colon)
- Date of initial diagnosis and TNM stage: May 15, 2025; T2N2aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 10, 2025 - peritoneum, bone, brain metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Weight loss, Fatigue, Early satiety, Change in bowel habits, Vomiting, Diarrhea
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Hypothyroidism
 - Migraine headaches
 - Thyroid disorder
 - Asthma
- Surgical history:
 - Appendectomy (2008)

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: No known familial syndromes

Social History

- Smoking: Never smoker
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Office administrator
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency

- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Penicillin (rash)

Physical Examination

- Vitals: BP 136/81, HR 81, RR 17, Temp 98.8°F, SpO2 96% on room air, Weight 163 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $13.8 \times 10^3/\mu\text{L}$, ANC: $11.2 \times 10^3/\mu\text{L}$
- Hgb: 14.9 g/dL, Platelets: $165 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.8 mg/dL, eGFR: 102 mL/min/1.73m²
- LFTs:
- AST: 61 U/L, ALT: 97 U/L
- Alkaline phosphatase: 154 U/L, Total bilirubin: 0.8 mg/dL
- CEA: 95.1 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: V600E
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 10, 2025):
- Primary ascending colon mass (4.7 cm), near-obstructing lesion
- peritoneal disease, osseous metastases, 2 brain metastases
- Other findings: Atherosclerotic calcifications, Hepatic steatosis
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (peritoneum, bone, brain)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 28543394

Date of Visit: June 03, 2025

Oncologist: Dr. Jennifer Rodriguez, MD

Chief Complaint

- Cramping, Diarrhea, Weight loss

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 04, 2025
- Site of primary tumor: Upper rectum (rectum)
- Date of initial diagnosis and TNM stage: April 04, 2025; T2N2aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 11, 2025 - bone, liver, brain metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Cramping, Diarrhea, Weight loss, Abdominal pain, Nausea, Rectal bleeding
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Chronic fatigue syndrome
 - Inflammatory bowel disease
- Surgical history:
 - Testicular surgery (2020)
 - Circumcision (2013)

Family History

- Colorectal or other cancers in first-degree relatives: Paternal uncle with prostate cancer
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Former smoker, quit 5 years ago (15 pack-year history)
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired teacher
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms

- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Penicillin (rash)

Physical Examination

- Vitals: BP 125/86, HR 98, RR 20, Temp 98.4°F, SpO2 98% on room air, Weight 201 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $9.5 \times 10^3/\mu\text{L}$, ANC: $6.5 \times 10^3/\mu\text{L}$
- Hgb: 15.0 g/dL, Platelets: $233 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.0 mg/dL, eGFR: 61 mL/min/1.73m²
- LFTs:
- AST: 26 U/L, ALT: 17 U/L
- Alkaline phosphatase: 92 U/L, Total bilirubin: 0.3 mg/dL
- CEA: 154.9 ng/mL (elevated)
- Molecular markers:
- RAS: NRAS Q61H
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 11, 2025):
- Primary upper rectum mass (4.7 cm), infiltrative mass
- 1 hepatic metastases (largest 2.9 cm), osseous metastases, 1 brain metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (bone, liver, brain)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 34694074

Date of Visit: June 11, 2025

Oncologist: Dr. Jennifer Rodriguez, MD

Chief Complaint

- Nausea, Weight loss, Cramping

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 20, 2025
- Site of primary tumor: Transverse colon (right colon)
- Date of initial diagnosis and TNM stage: April 20, 2025; T4aN1bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 10, 2025 - brain, liver, lung metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Nausea, Weight loss, Cramping, Rectal bleeding, Fatigue
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Hypertension
 - Osteoporosis
 - Dementia (early stage)
- Surgical history:
 - Vasectomy (2004)
 - Prostate biopsy (2012)
 - Carotid endarterectomy (2022)

Family History

- Colorectal or other cancers in first-degree relatives: Father with colon cancer at age 68
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Current smoker (10 pack-years)
- Alcohol use: Rare alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Engineer
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing

- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Amlodipine 5mg daily
- Multivitamin daily
- Allergies: Penicillin (rash), Shellfish (hives)

Physical Examination

- Vitals: BP 125/71, HR 100, RR 19, Temp 98.9°F, SpO2 99% on room air, Weight 180 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $13.5 \times 10^3/\mu\text{L}$, ANC: $6.9 \times 10^3/\mu\text{L}$
- Hgb: 9.9 g/dL, Platelets: $201 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.2 mg/dL, eGFR: 95 mL/min/1.73m²
- LFTs:
- AST: 42 U/L, ALT: 73 U/L
- Alkaline phosphatase: 219 U/L, Total bilirubin: 1.3 mg/dL
- CEA: 105.3 ng/mL (elevated)
- Molecular markers:
- RAS: NRAS Q61H
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 10, 2025):
- Primary transverse colon mass (6.1 cm), circumferential thickening
- 5 hepatic metastases (largest 4.4 cm), 1 pulmonary nodules (largest 2.1 cm), 2 brain metastases
- Other findings: Hepatic steatosis
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (brain, liver, lung)
- Disease status: Newly diagnosed, treatment-naïve

- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 76190966

Date of Visit: May 28, 2025

Oncologist: Dr. Michael Chen, MD

Chief Complaint

- Abdominal pain, Rectal bleeding, Early satiety

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 02, 2025
- Site of primary tumor: Cecum (right colon)
- Date of initial diagnosis and TNM stage: April 02, 2025; T3N0M1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 14, 2025 - bone, lung metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Abdominal pain, Rectal bleeding, Early satiety
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Stroke history
 - Fibromyalgia
 - Chronic kidney disease stage 2-3
 - Chronic kidney disease
 - Coronary artery disease
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Current smoker (10 pack-years)
- Alcohol use: Rare alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Engineer
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing

- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Multivitamin daily
- Allergies: Sulfa drugs (GI upset)

Physical Examination

- Vitals: BP 129/85, HR 76, RR 19, Temp 99.1°F, SpO2 97% on room air, Weight 159 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $14.4 \times 10^3/\mu\text{L}$, ANC: $10.6 \times 10^3/\mu\text{L}$
- Hgb: 13.2 g/dL, Platelets: $357 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.3 mg/dL, eGFR: 63 mL/min/1.73m²
- LFTs:
- AST: 87 U/L, ALT: 45 U/L
- Alkaline phosphatase: 46 U/L, Total bilirubin: 1.2 mg/dL
- CEA: 44.7 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G12D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 14, 2025):
- Primary cecum mass (6.7 cm), circumferential thickening
- 5 pulmonary nodules (largest 3.5 cm), osseous metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (bone, lung)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 46385580

Date of Visit: June 08, 2025

Oncologist: Dr. David Thompson, MD

Chief Complaint

- Nausea, Fatigue, Bloating

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 18, 2025
- Site of primary tumor: Mid rectum (rectum)
- Date of initial diagnosis and TNM stage: April 18, 2025; T2N2bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 10, 2025 - lung metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Nausea, Fatigue, Bloating, Constipation, Weight loss, Change in bowel habits
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Chronic kidney disease stage 2-3
 - Type 2 diabetes
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Father with colon cancer at age 68
- Known familial syndromes: No known familial syndromes

Social History

- Smoking: Former smoker, quit 5 years ago (15 pack-year history)
- Alcohol use: Social drinking, 2-3 drinks per week
- Living situation/support: Lives with spouse, strong family support
- Occupation: Office administrator
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness

- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Glipizide 5mg daily
- Multivitamin daily
- Allergies: Penicillin (rash), Shellfish (hives)

Physical Examination

- Vitals: BP 123/70, HR 75, RR 15, Temp 98.1°F, SpO2 96% on room air, Weight 189 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $4.0 \times 10^3/\mu\text{L}$, ANC: $10.2 \times 10^3/\mu\text{L}$
- Hgb: 9.5 g/dL, Platelets: $324 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.9 mg/dL, eGFR: 99 mL/min/1.73m²
- LFTs:
- AST: 65 U/L, ALT: 106 U/L
- Alkaline phosphatase: 104 U/L, Total bilirubin: 0.5 mg/dL
- CEA: 120.5 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G12D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 10, 2025):
- Primary mid rectum mass (7.5 cm), polypoid mass
- 4 pulmonary nodules (largest 3.8 cm)
- Other findings: Degenerative spine changes, Atherosclerotic calcifications
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lung)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 71406421

Date of Visit: June 02, 2025

Oncologist: Dr. David Thompson, MD

Chief Complaint

- Diarrhea, Bloating, Vomiting

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 12, 2025
- Site of primary tumor: Sigmoid colon (left colon)
- Date of initial diagnosis and TNM stage: May 12, 2025; T3N2aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 07, 2025 - bone metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Diarrhea, Bloating, Vomiting, Constipation
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Type 1 diabetes
- Surgical history:
 - Ovarian cyst removal (1996)
 - Hysterectomy (2007)

Family History

- Colorectal or other cancers in first-degree relatives: No known family history of cancer
- Known familial syndromes: No known familial syndromes

Social History

- Smoking: Former smoker, quit 10 years ago (20 pack-year history)
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Sales manager
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness

- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Penicillin (rash)

Physical Examination

- Vitals: BP 116/73, HR 94, RR 16, Temp 99.2°F, SpO2 98% on room air, Weight 190 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $4.9 \times 10^3/\mu\text{L}$, ANC: $11.2 \times 10^3/\mu\text{L}$
- Hgb: 10.2 g/dL, Platelets: $388 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.8 mg/dL, eGFR: 35 mL/min/1.73m²
- LFTs:
- AST: 83 U/L, ALT: 78 U/L
- Alkaline phosphatase: 283 U/L, Total bilirubin: 0.7 mg/dL
- CEA: 164.8 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G13D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 07, 2025):
Primary sigmoid colon mass (6.6 cm), near-obstructing lesion
osseous metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (bone)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 48727925

Date of Visit: May 26, 2025

Oncologist: Dr. Sarah Mitchell, MD

Chief Complaint

- Diarrhea, Change in bowel habits, Constipation

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 06, 2025
- Site of primary tumor: Transverse colon (right colon)
- Date of initial diagnosis and TNM stage: April 06, 2025; T2N2aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 08, 2025 - lung metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Diarrhea, Change in bowel habits, Constipation
- Performance status: ECOG 0
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Osteoporosis
 - Stroke history
 - Chronic kidney disease
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Current smoker (10 pack-years)
- Alcohol use: Former alcohol use, quit 5 years ago
- Living situation/support: Lives with spouse, strong family support
- Occupation: Small business owner
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms

- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Multivitamin daily
- Allergies: NKDA (No Known Drug Allergies)

Physical Examination

- Vitals: BP 136/78, HR 90, RR 14, Temp 98.8°F, SpO2 99% on room air, Weight 159 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $11.2 \times 10^3/\mu\text{L}$, ANC: $2.2 \times 10^3/\mu\text{L}$
- Hgb: 14.0 g/dL, Platelets: $244 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.8 mg/dL, eGFR: 74 mL/min/1.73m²
- LFTs:
- AST: 30 U/L, ALT: 33 U/L
- Alkaline phosphatase: 68 U/L, Total bilirubin: 0.4 mg/dL
- CEA: 145.8 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G12D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 08, 2025):
- Primary transverse colon mass (3.9 cm), infiltrative mass
- 3 pulmonary nodules (largest 2.9 cm)
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lung)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 18228455

Date of Visit: June 13, 2025

Oncologist: Dr. Michael Chen, MD

Chief Complaint

- Fatigue, Change in bowel habits, Weight loss

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 18, 2025
- Site of primary tumor: Hepatic flexure (right colon)
- Date of initial diagnosis and TNM stage: April 18, 2025; T4bN1aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 14, 2025 - lung, bone metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Fatigue, Change in bowel habits, Weight loss, Vomiting, Constipation, Rectal bleeding
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
- Chronic fatigue syndrome
- Asthma
- Surgical history:
- Breast biopsy (2004)

Family History

- Colorectal or other cancers in first-degree relatives: Father with colon cancer at age 68
- Known familial syndromes: No known familial syndromes

Social History

- Smoking: Former smoker, quit 10 years ago (20 pack-year history)
- Alcohol use: Social drinking, 2-3 drinks per week
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired mechanic
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness

- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Sulfa drugs (GI upset)

Physical Examination

- Vitals: BP 124/73, HR 76, RR 20, Temp 98.9°F, SpO2 97% on room air, Weight 165 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $11.6 \times 10^3/\mu\text{L}$, ANC: $4.1 \times 10^3/\mu\text{L}$
- Hgb: 12.5 g/dL, Platelets: $428 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.8 mg/dL, eGFR: 71 mL/min/1.73m²
- LFTs:
- AST: 26 U/L, ALT: 25 U/L
- Alkaline phosphatase: 54 U/L, Total bilirubin: 0.8 mg/dL
- CEA: 69.6 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSI-H

- Imaging:

- CT chest/abdomen/pelvis (June 14, 2025):
- Primary hepatic flexure mass (4.3 cm), infiltrative mass
- 5 pulmonary nodules (largest 4.1 cm), osseous metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lung, bone)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 88044380

Date of Visit: May 27, 2025

Oncologist: Dr. David Thompson, MD

Chief Complaint

- Constipation, Abdominal pain, Rectal bleeding

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 20, 2025
- Site of primary tumor: Splenic flexure (left colon)
- Date of initial diagnosis and TNM stage: May 20, 2025; T3N1bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 16, 2025 - brain metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Constipation, Abdominal pain, Rectal bleeding
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Hypertension
 - Depression
 - Hypothyroidism
 - Cataracts
 - Peripheral artery disease
- Surgical history:
 - Prostate biopsy (2007)
 - Testicular surgery (1992)

Family History

- Colorectal or other cancers in first-degree relatives: Paternal uncle with prostate cancer
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Never smoker
- Alcohol use: Rare alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired mechanic
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath

- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Amlodipine 5mg daily
- Multivitamin daily
- Allergies: NKDA (No Known Drug Allergies)

Physical Examination

- Vitals: BP 139/78, HR 86, RR 15, Temp 98.5°F, SpO2 100% on room air, Weight 173 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $5.1 \times 10^3/\mu\text{L}$, ANC: $6.9 \times 10^3/\mu\text{L}$
- Hgb: 13.7 g/dL, Platelets: $173 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.2 mg/dL, eGFR: 116 mL/min/1.73m²
- LFTs:
- AST: 15 U/L, ALT: 19 U/L
- Alkaline phosphatase: 76 U/L, Total bilirubin: 0.8 mg/dL
- CEA: 198.2 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G12D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 16, 2025):
- Primary splenic flexure mass (6.8 cm), polypoid mass
- 4 brain metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (brain)
- Disease status: Newly diagnosed, treatment-naïve

- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 11800359

Date of Visit: June 17, 2025

Oncologist: Dr. Jennifer Rodriguez, MD

Chief Complaint

- Change in bowel habits, Weight loss, Bloating

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 20, 2025
- Site of primary tumor: Descending colon (left colon)
- Date of initial diagnosis and TNM stage: April 20, 2025; T3N1bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 04, 2025 - bone, lymph nodes, lung metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Change in bowel habits, Weight loss, Bloating, Rectal bleeding
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Osteoarthritis
 - Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Former smoker, quit 5 years ago (15 pack-year history)
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Sales manager
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes

- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
 - None
- Allergies: Penicillin (rash), Shellfish (hives)

Physical Examination

- Vitals: BP 135/89, HR 79, RR 14, Temp 99.3°F, SpO2 100% on room air, Weight 163 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
 - WBC: $10.2 \times 10^3/\mu\text{L}$, ANC: $11.6 \times 10^3/\mu\text{L}$
 - Hgb: 12.7 g/dL, Platelets: $390 \times 10^3/\mu\text{L}$
- Chemistry:
 - Creatinine: 1.5 mg/dL, eGFR: 67 mL/min/1.73m²
- LFTs:
 - AST: 42 U/L, ALT: 110 U/L
 - Alkaline phosphatase: 170 U/L, Total bilirubin: 1.6 mg/dL
- CEA: 16.1 ng/mL (elevated)
- Molecular markers:
 - RAS: NRAS Q61H
 - BRAF: Wild-type
 - MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 04, 2025):
 - Primary descending colon mass (5.1 cm), annular narrowing
 - 4 pulmonary nodules (largest 3.9 cm), enlarged lymph nodes (largest 3.6 cm), osseous metastases
 - Other findings: Atherosclerotic calcifications, Renal cysts, Hiatal hernia
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (bone, lymph nodes, lung)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 66328674

Date of Visit: June 17, 2025

Oncologist: Dr. Lisa Park, MD

Chief Complaint

- Cramping, Change in bowel habits, Diarrhea

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 11, 2025
- Site of primary tumor: Ascending colon (right colon)
- Date of initial diagnosis and TNM stage: May 11, 2025; T3N0M1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 05, 2025 - brain, peritoneum metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Cramping, Change in bowel habits, Diarrhea, Abdominal pain
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - ADHD
 - Endometriosis
 - Chronic fatigue syndrome
- Surgical history:
 - Vasectomy (2014)
 - Appendectomy (2020)

Family History

- Colorectal or other cancers in first-degree relatives: Paternal uncle with prostate cancer
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Never smoker
- Alcohol use: Former alcohol use, quit 5 years ago
- Living situation/support: Lives with spouse, strong family support
- Occupation: Engineer
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency

- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: NKDA (No Known Drug Allergies)

Physical Examination

- Vitals: BP 116/76, HR 78, RR 15, Temp 99.4°F, SpO2 100% on room air, Weight 158 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $11.6 \times 10^3/\mu\text{L}$, ANC: $3.5 \times 10^3/\mu\text{L}$
- Hgb: 14.5 g/dL, Platelets: $370 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.0 mg/dL, eGFR: 100 mL/min/1.73m²
- LFTs:
- AST: 15 U/L, ALT: 11 U/L
- Alkaline phosphatase: 96 U/L, Total bilirubin: 0.3 mg/dL
- CEA: 199.9 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G13D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 05, 2025):
Primary ascending colon mass (7.5 cm), polypoid mass
peritoneal disease, 1 brain metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (brain, peritoneum)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 88760810

Date of Visit: May 23, 2025

Oncologist: Dr. Sarah Mitchell, MD

Chief Complaint

- Change in bowel habits, Cramping, Nausea

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 15, 2025
- Site of primary tumor: Transverse colon (right colon)
- Date of initial diagnosis and TNM stage: April 15, 2025; T3N1bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 16, 2025 - bone, lymph nodes, liver metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Change in bowel habits, Cramping, Nausea, Early satiety, Abdominal pain
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - None
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: No known family history of cancer
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Former smoker, quit 5 years ago (15 pack-year history)
- Alcohol use: Social drinking, 2-3 drinks per week
- Living situation/support: Lives with spouse, strong family support
- Occupation: Nurse
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes

- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Sulfa drugs (GI upset)

Physical Examination

- Vitals: BP 124/71, HR 95, RR 15, Temp 99.2°F, SpO2 97% on room air, Weight 162 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $7.0 \times 10^3/\mu\text{L}$, ANC: $6.1 \times 10^3/\mu\text{L}$
- Hgb: 15.9 g/dL, Platelets: $179 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.8 mg/dL, eGFR: 118 mL/min/1.73m²
- LFTs:
- AST: 34 U/L, ALT: 23 U/L
- Alkaline phosphatase: 63 U/L, Total bilirubin: 1.0 mg/dL
- CEA: 112.0 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSI-H

- Imaging:

- CT chest/abdomen/pelvis (June 16, 2025):
- Primary transverse colon mass (6.3 cm), polypoid mass
- 8 hepatic metastases (largest 6.4 cm), enlarged lymph nodes (largest 2.6 cm), osseous metastases
- Other findings: Pulmonary nodules (likely benign), Atherosclerotic calcifications, Diverticulosis
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (bone, lymph nodes, liver)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 92533443

Date of Visit: May 28, 2025

Oncologist: Dr. Lisa Park, MD

Chief Complaint

- Constipation, Fatigue, Rectal bleeding

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 13, 2025
- Site of primary tumor: Cecum (right colon)
- Date of initial diagnosis and TNM stage: April 13, 2025; T4aN1bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 12, 2025 - lung, bone, brain metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Constipation, Fatigue, Rectal bleeding, Cramping, Abdominal pain, Bloating
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - None
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: No known family history of cancer
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Current smoker (10 pack-years)
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Nurse
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes

- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Penicillin (rash), Shellfish (hives)

Physical Examination

- Vitals: BP 133/74, HR 85, RR 15, Temp 98.8°F, SpO2 97% on room air, Weight 186 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $3.7 \times 10^3/\mu\text{L}$, ANC: $7.1 \times 10^3/\mu\text{L}$
- Hgb: 15.8 g/dL, Platelets: $255 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.8 mg/dL, eGFR: 115 mL/min/1.73m²
- LFTs:
- AST: 24 U/L, ALT: 20 U/L
- Alkaline phosphatase: 61 U/L, Total bilirubin: 0.6 mg/dL
- CEA: 140.6 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G13D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 12, 2025):
- Primary cecum mass (7.2 cm), circumferential thickening
- 5 pulmonary nodules (largest 4.1 cm), osseous metastases, 2 brain metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lung, bone, brain)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 52965770

Date of Visit: May 21, 2025

Oncologist: Dr. Lisa Park, MD

Chief Complaint

- Constipation, Weight loss, Cramping

History of Present Illness

- Date of initial colorectal cancer diagnosis: March 27, 2025
- Site of primary tumor: Ascending colon (right colon)
- Date of initial diagnosis and TNM stage: March 27, 2025; T2N1aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 06, 2025 - peritoneum, bone, lung metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Constipation, Weight loss, Cramping, Diarrhea, Nausea, Change in bowel habits
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - None
- Surgical history:
 - Vasectomy (1999)
 - Appendectomy (2002)
 - Cholecystectomy (2014)
 - Circumcision (2014)

Family History

- Colorectal or other cancers in first-degree relatives: Mother with breast cancer at age 72
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Never smoker
- Alcohol use: Former alcohol use, quit 5 years ago
- Living situation/support: Lives with spouse, strong family support
- Occupation: Nurse
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing

- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Penicillin (rash)

Physical Examination

- Vitals: BP 112/77, HR 90, RR 18, Temp 98.5°F, SpO2 99% on room air, Weight 131 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $4.4 \times 10^3/\mu\text{L}$, ANC: $4.7 \times 10^3/\mu\text{L}$
- Hgb: 13.7 g/dL, Platelets: $305 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.8 mg/dL, eGFR: 111 mL/min/1.73m²
- LFTs:
- AST: 33 U/L, ALT: 20 U/L
- Alkaline phosphatase: 58 U/L, Total bilirubin: 0.5 mg/dL
- CEA: 51.4 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G13D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 06, 2025):
- Primary ascending colon mass (6.6 cm), infiltrative mass
- 5 pulmonary nodules (largest 4.1 cm), peritoneal disease, osseous metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (peritoneum, bone, lung)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 42016742

Date of Visit: June 05, 2025

Oncologist: Dr. Lisa Park, MD

Chief Complaint

- Fatigue, Bloating, Cramping

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 18, 2025
- Site of primary tumor: Mid rectum (rectum)
- Date of initial diagnosis and TNM stage: April 18, 2025; T4aN0M1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 16, 2025 - peritoneum metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Fatigue, Bloating, Cramping, Diarrhea, Weight loss, Abdominal pain
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Coronary artery disease
 - Psoriasis
 - GERD
 - Type 2 diabetes
 - Dementia (early stage)
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Never smoker
- Alcohol use: Rare alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Office administrator
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing

- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Metformin 1000mg twice daily
- Multivitamin daily
- Allergies: Sulfa drugs (GI upset)

Physical Examination

- Vitals: BP 115/78, HR 83, RR 14, Temp 99.0°F, SpO2 99% on room air, Weight 192 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $5.7 \times 10^3/\mu\text{L}$, ANC: $6.0 \times 10^3/\mu\text{L}$
- Hgb: 15.9 g/dL, Platelets: $376 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.2 mg/dL, eGFR: 92 mL/min/1.73m²
- LFTs:
- AST: 15 U/L, ALT: 26 U/L
- Alkaline phosphatase: 131 U/L, Total bilirubin: 0.9 mg/dL
- CEA: 194.1 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G12D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 16, 2025):
- Primary mid rectum mass (4.4 cm), infiltrative mass
- peritoneal disease
- Other findings: Cholelithiasis
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (peritoneum)
- Disease status: Newly diagnosed, treatment-naïve

- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 37490902

Date of Visit: June 07, 2025

Oncologist: Dr. Michael Chen, MD

Chief Complaint

- Weight loss, Abdominal pain, Early satiety

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 30, 2025
- Site of primary tumor: Upper rectum (rectum)
- Date of initial diagnosis and TNM stage: May 30, 2025; T2N2bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 09, 2025 - brain, peritoneum, bone metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Weight loss, Abdominal pain, Early satiety, Change in bowel habits, Diarrhea
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Sleep apnea
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: No known familial syndromes

Social History

- Smoking: Former smoker, quit 5 years ago (15 pack-year history)
- Alcohol use: Former alcohol use, quit 5 years ago
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired teacher
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes

- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Multivitamin daily
- Allergies: Shellfish (mild GI upset)

Physical Examination

- Vitals: BP 122/80, HR 71, RR 15, Temp 99.4°F, SpO2 99% on room air, Weight 167 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $12.8 \times 10^3/\mu\text{L}$, ANC: $9.8 \times 10^3/\mu\text{L}$
- Hgb: 10.6 g/dL, Platelets: $432 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.7 mg/dL, eGFR: 44 mL/min/1.73m²
- LFTs:
- AST: 48 U/L, ALT: 89 U/L
- Alkaline phosphatase: 244 U/L, Total bilirubin: 2.0 mg/dL
- CEA: 123.4 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 09, 2025):
- Primary upper rectum mass (4.7 cm), near-obstructing lesion
- peritoneal disease, osseous metastases, 4 brain metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (brain, peritoneum, bone)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 80132218

Date of Visit: May 19, 2025

Oncologist: Dr. David Thompson, MD

Chief Complaint

- Rectal bleeding, Weight loss, Early satiety

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 10, 2025
- Site of primary tumor: Descending colon (left colon)
- Date of initial diagnosis and TNM stage: May 10, 2025; T2N2aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 03, 2025 - brain, lymph nodes, peritoneum metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Rectal bleeding, Weight loss, Early satiety, Abdominal pain, Nausea, Diarrhea
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Celiac disease
 - Chronic back pain
 - Hyperlipidemia
 - Psoriasis
- Surgical history:
 - Prostate biopsy (1991)

Family History

- Colorectal or other cancers in first-degree relatives: Paternal uncle with prostate cancer
- Known familial syndromes: No known familial syndromes

Social History

- Smoking: Former smoker, quit 5 years ago (15 pack-year history)
- Alcohol use: Former alcohol use, quit 5 years ago
- Living situation/support: Lives with spouse, strong family support
- Occupation: Office administrator
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency

- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Simvastatin 40mg daily
- Allergies: NKDA (No Known Drug Allergies)

Physical Examination

- Vitals: BP 136/88, HR 97, RR 15, Temp 98.1°F, SpO2 100% on room air, Weight 159 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $5.8 \times 10^3/\mu\text{L}$, ANC: $3.7 \times 10^3/\mu\text{L}$
- Hgb: 13.5 g/dL, Platelets: $151 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.8 mg/dL, eGFR: 89 mL/min/1.73m²
- LFTs:
- AST: 58 U/L, ALT: 41 U/L
- Alkaline phosphatase: 173 U/L, Total bilirubin: 2.6 mg/dL
- CEA: 134.7 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G12D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 03, 2025):
- Primary descending colon mass (4.5 cm), annular narrowing
- peritoneal disease, enlarged lymph nodes (largest 2.0 cm), 1 brain metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (brain, lymph nodes, peritoneum)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 49300510

Date of Visit: May 27, 2025

Oncologist: Dr. Michael Chen, MD

Chief Complaint

- Early satiety, Vomiting, Diarrhea

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 05, 2025
- Site of primary tumor: Descending colon (left colon)
- Date of initial diagnosis and TNM stage: April 05, 2025; T4bN2aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 06, 2025 - bone metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Early satiety, Vomiting, Diarrhea, Nausea, Cramping, Weight loss
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Hyperlipidemia
 - Macular degeneration
 - Sleep apnea
 - Chronic kidney disease
- Surgical history:
 - Appendectomy (2002)

Family History

- Colorectal or other cancers in first-degree relatives: Mother with breast cancer at age 72
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Never smoker
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired teacher
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency

- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Atorvastatin 20mg daily
- Multivitamin daily
- Allergies: Penicillin (rash), Shellfish (hives)

Physical Examination

- Vitals: BP 134/73, HR 82, RR 14, Temp 98.7°F, SpO2 96% on room air, Weight 161 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $6.1 \times 10^3/\mu\text{L}$, ANC: $4.2 \times 10^3/\mu\text{L}$
- Hgb: 14.0 g/dL, Platelets: $192 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.6 mg/dL, eGFR: 79 mL/min/1.73m²
- LFTs:
- AST: 17 U/L, ALT: 52 U/L
- Alkaline phosphatase: 207 U/L, Total bilirubin: 0.6 mg/dL
- CEA: 58.5 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 06, 2025):
- Primary descending colon mass (5.7 cm), near-obstructing lesion
- osseous metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (bone)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 16151860

Date of Visit: May 24, 2025

Oncologist: Dr. Michael Chen, MD

Chief Complaint

- Weight loss, Rectal bleeding, Early satiety

History of Present Illness

- Date of initial colorectal cancer diagnosis: March 31, 2025
- Site of primary tumor: Hepatic flexure (right colon)
- Date of initial diagnosis and TNM stage: March 31, 2025; T2N0M1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 04, 2025 - brain, lymph nodes, liver metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Weight loss, Rectal bleeding, Early satiety, Vomiting, Cramping, Bloating
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Fibromyalgia
 - Anxiety disorder
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Father with colon cancer at age 68
- Known familial syndromes: No known familial syndromes

Social History

- Smoking: Former smoker, quit 10 years ago (20 pack-year history)
- Alcohol use: Rare alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Engineer
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness

- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: NKDA (No Known Drug Allergies)

Physical Examination

- Vitals: BP 128/90, HR 94, RR 16, Temp 98.5°F, SpO2 96% on room air, Weight 147 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $9.7 \times 10^3/\mu\text{L}$, ANC: $2.1 \times 10^3/\mu\text{L}$
- Hgb: 13.8 g/dL, Platelets: $412 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.0 mg/dL, eGFR: 70 mL/min/1.73m²
- LFTs:
- AST: 26 U/L, ALT: 34 U/L
- Alkaline phosphatase: 135 U/L, Total bilirubin: 1.0 mg/dL
- CEA: 182.0 ng/mL (elevated)
- Molecular markers:
- RAS: NRAS Q61H
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 04, 2025):
- Primary hepatic flexure mass (3.4 cm), circumferential thickening
- 2 hepatic metastases (largest 5.2 cm), enlarged lymph nodes (largest 3.5 cm), 3 brain metastases
- Other findings: Diverticulosis, Atherosclerotic calcifications
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (brain, lymph nodes, liver)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 65281705

Date of Visit: May 20, 2025

Oncologist: Dr. David Thompson, MD

Chief Complaint

- Early satiety, Nausea, Change in bowel habits

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 06, 2025
- Site of primary tumor: Lower rectum (rectum)
- Date of initial diagnosis and TNM stage: May 06, 2025; T4aN2aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 09, 2025 - bone metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Early satiety, Nausea, Change in bowel habits
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Coronary artery disease
 - Stroke history
 - Depression
 - Type 2 diabetes
 - Fibromyalgia
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Paternal uncle with prostate cancer
- Known familial syndromes: No known familial syndromes

Social History

- Smoking: Former smoker, quit 10 years ago (20 pack-year history)
- Alcohol use: Social drinking, 2-3 drinks per week
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired mechanic
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing

- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Glipizide 5mg daily
- Multivitamin daily
- Allergies: Shellfish (mild GI upset)

Physical Examination

- Vitals: BP 134/73, HR 80, RR 19, Temp 98.5°F, SpO2 97% on room air, Weight 129 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $4.4 \times 10^3/\mu\text{L}$, ANC: $2.6 \times 10^3/\mu\text{L}$
- Hgb: 14.4 g/dL, Platelets: $203 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.0 mg/dL, eGFR: 110 mL/min/1.73m²
- LFTs:
- AST: 40 U/L, ALT: 33 U/L
- Alkaline phosphatase: 78 U/L, Total bilirubin: 1.1 mg/dL
- CEA: 193.1 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G12D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 09, 2025):
- Primary lower rectum mass (4.7 cm), circumferential thickening
- osseous metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (bone)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time

- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 82646700

Date of Visit: May 28, 2025

Oncologist: Dr. Sarah Mitchell, MD

Chief Complaint

- Weight loss, Bloating, Vomiting

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 20, 2025
- Site of primary tumor: Sigmoid colon (left colon)
- Date of initial diagnosis and TNM stage: April 20, 2025; T3N1bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 12, 2025 - lung metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Weight loss, Bloating, Vomiting, Nausea, Change in bowel habits
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - ADHD
 - Endometriosis
 - Chronic back pain
 - Depression
- Surgical history:
 - Cholecystectomy (2024)

Family History

- Colorectal or other cancers in first-degree relatives: Father with colon cancer at age 68
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Never smoker
- Alcohol use: Former alcohol use, quit 5 years ago
- Living situation/support: Lives with spouse, strong family support
- Occupation: Small business owner
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency

- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Shellfish (mild GI upset)

Physical Examination

- Vitals: BP 128/79, HR 70, RR 19, Temp 98.1°F, SpO2 100% on room air, Weight 198 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $6.6 \times 10^3/\mu\text{L}$, ANC: $3.1 \times 10^3/\mu\text{L}$
- Hgb: 15.9 g/dL, Platelets: $218 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.2 mg/dL, eGFR: 72 mL/min/1.73m²
- LFTs:
- AST: 20 U/L, ALT: 28 U/L
- Alkaline phosphatase: 53 U/L, Total bilirubin: 0.8 mg/dL
- CEA: 132.6 ng/mL (elevated)
- Molecular markers:
- RAS: NRAS Q61H
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 12, 2025):
- Primary sigmoid colon mass (6.0 cm), annular narrowing
- 3 pulmonary nodules (largest 2.6 cm)
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lung)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 44631066

Date of Visit: June 11, 2025

Oncologist: Dr. Jennifer Rodriguez, MD

Chief Complaint

- Constipation, Rectal bleeding, Early satiety

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 16, 2025
- Site of primary tumor: Ascending colon (right colon)
- Date of initial diagnosis and TNM stage: May 16, 2025; T2N0M1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 10, 2025 - lymph nodes, brain metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Constipation, Rectal bleeding, Early satiety, Vomiting, Fatigue
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Macular degeneration
 - COPD
- Surgical history:
 - Pacemaker insertion (2016)

Family History

- Colorectal or other cancers in first-degree relatives: Father with colon cancer at age 68
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Never smoker
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired teacher
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness

- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Multivitamin daily
- Allergies: Penicillin (rash), Shellfish (hives)

Physical Examination

- Vitals: BP 118/89, HR 91, RR 18, Temp 99.0°F, SpO2 99% on room air, Weight 169 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $13.6 \times 10^3/\mu\text{L}$, ANC: $11.5 \times 10^3/\mu\text{L}$
- Hgb: 12.3 g/dL, Platelets: $132 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.4 mg/dL, eGFR: 110 mL/min/1.73m²
- LFTs:
- AST: 29 U/L, ALT: 31 U/L
- Alkaline phosphatase: 120 U/L, Total bilirubin: 2.3 mg/dL
- CEA: 95.9 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSI-H

- Imaging:

- CT chest/abdomen/pelvis (June 10, 2025):
Primary ascending colon mass (7.4 cm), infiltrative mass
enlarged lymph nodes (largest 3.3 cm), 4 brain metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lymph nodes, brain)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 28786181

Date of Visit: May 22, 2025

Oncologist: Dr. Jennifer Rodriguez, MD

Chief Complaint

- Bloating, Change in bowel habits, Cramping

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 08, 2025
- Site of primary tumor: Sigmoid colon (left colon)
- Date of initial diagnosis and TNM stage: April 08, 2025; T3N2aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 09, 2025 - lymph nodes, bone metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Bloating, Change in bowel habits, Cramping, Rectal bleeding
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Endometriosis
 - Thyroid disorder
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Father with colon cancer at age 68
- Known familial syndromes: No known familial syndromes

Social History

- Smoking: Never smoker
- Alcohol use: Social drinking, 2-3 drinks per week
- Living situation/support: Lives with spouse, strong family support
- Occupation: Sales manager
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness

- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Penicillin (rash)

Physical Examination

- Vitals: BP 113/80, HR 76, RR 20, Temp 99.4°F, SpO2 97% on room air, Weight 188 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $5.9 \times 10^3/\mu\text{L}$, ANC: $3.5 \times 10^3/\mu\text{L}$
- Hgb: 15.0 g/dL, Platelets: $310 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.8 mg/dL, eGFR: 64 mL/min/1.73m²
- LFTs:
- AST: 37 U/L, ALT: 27 U/L
- Alkaline phosphatase: 131 U/L, Total bilirubin: 1.1 mg/dL
- CEA: 145.3 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 09, 2025):
- Primary sigmoid colon mass (7.6 cm), polypoid mass
- enlarged lymph nodes (largest 3.6 cm), osseous metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lymph nodes, bone)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 59145335

Date of Visit: June 01, 2025

Oncologist: Dr. Sarah Mitchell, MD

Chief Complaint

- Rectal bleeding, Cramping, Bloating

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 21, 2025
- Site of primary tumor: Ascending colon (right colon)
- Date of initial diagnosis and TNM stage: May 21, 2025; T4aN1bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 09, 2025 - brain metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Rectal bleeding, Cramping, Bloating, Diarrhea, Fatigue, Abdominal pain
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - ADHD
 - Depression
 - Chronic fatigue syndrome
- Surgical history:
 - Appendectomy (1998)

Family History

- Colorectal or other cancers in first-degree relatives: Mother with breast cancer at age 72
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Former smoker, quit 10 years ago (20 pack-year history)
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Nurse
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms

- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Sulfa drugs (GI upset)

Physical Examination

- Vitals: BP 125/80, HR 98, RR 18, Temp 98.3°F, SpO2 96% on room air, Weight 170 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $11.5 \times 10^3/\mu\text{L}$, ANC: $5.3 \times 10^3/\mu\text{L}$
- Hgb: 15.5 g/dL, Platelets: $261 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.2 mg/dL, eGFR: 60 mL/min/1.73m²
- LFTs:
- AST: 15 U/L, ALT: 22 U/L
- Alkaline phosphatase: 44 U/L, Total bilirubin: 0.9 mg/dL
- CEA: 113.0 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G12D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 09, 2025):
- Primary ascending colon mass (5.8 cm), annular narrowing
- 4 brain metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (brain)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 50203449

Date of Visit: June 12, 2025

Oncologist: Dr. Jennifer Rodriguez, MD

Chief Complaint

- Diarrhea, Rectal bleeding, Vomiting

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 02, 2025
- Site of primary tumor: Splenic flexure (left colon)
- Date of initial diagnosis and TNM stage: May 02, 2025; T3N2aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 15, 2025 - lung, lymph nodes metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Diarrhea, Rectal bleeding, Vomiting, Bloating, Constipation
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - None
- Surgical history:
 - Breast biopsy (2004)

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: No known familial syndromes

Social History

- Smoking: Former smoker, quit 10 years ago (20 pack-year history)
- Alcohol use: Social drinking, 2-3 drinks per week
- Living situation/support: Lives with spouse, strong family support
- Occupation: Office administrator
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes

- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: NKDA (No Known Drug Allergies)

Physical Examination

- Vitals: BP 115/77, HR 83, RR 17, Temp 99.2°F, SpO2 100% on room air, Weight 197 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $6.6 \times 10^3/\mu\text{L}$, ANC: $2.2 \times 10^3/\mu\text{L}$
- Hgb: 14.6 g/dL, Platelets: $412 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.8 mg/dL, eGFR: 94 mL/min/1.73m²
- LFTs:
- AST: 15 U/L, ALT: 25 U/L
- Alkaline phosphatase: 73 U/L, Total bilirubin: 0.8 mg/dL
- CEA: 25.5 ng/mL (elevated)
- Molecular markers:
- RAS: NRAS Q61H
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 15, 2025):
- Primary splenic flexure mass (6.1 cm), polypoid mass
- 5 pulmonary nodules (largest 3.6 cm), enlarged lymph nodes (largest 3.2 cm)
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lung, lymph nodes)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 40486227

Date of Visit: May 30, 2025

Oncologist: Dr. Sarah Mitchell, MD

Chief Complaint

- Diarrhea, Early satiety, Weight loss

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 27, 2025
- Site of primary tumor: Upper rectum (rectum)
- Date of initial diagnosis and TNM stage: April 27, 2025; T2N2bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 10, 2025 - lung, liver, bone metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Diarrhea, Early satiety, Weight loss, Nausea, Fatigue, Vomiting
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Hypothyroidism
 - Type 2 diabetes
 - Hypertension
 - Inflammatory bowel disease
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Paternal uncle with prostate cancer
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Former smoker, quit 10 years ago (20 pack-year history)
- Alcohol use: Rare alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired teacher
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency

- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Metoprolol 50mg twice daily
- Metformin 1000mg twice daily
- Multivitamin daily
- Allergies: NKDA (No Known Drug Allergies)

Physical Examination

- Vitals: BP 120/79, HR 95, RR 14, Temp 98.9°F, SpO2 97% on room air, Weight 126 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $9.2 \times 10^3/\mu\text{L}$, ANC: $6.2 \times 10^3/\mu\text{L}$
- Hgb: 15.2 g/dL, Platelets: $441 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.1 mg/dL, eGFR: 109 mL/min/1.73m²
- LFTs:
- AST: 30 U/L, ALT: 34 U/L
- Alkaline phosphatase: 143 U/L, Total bilirubin: 0.8 mg/dL
- CEA: 75.3 ng/mL (elevated)
- Molecular markers:
- RAS: NRAS Q61H
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 10, 2025):
- Primary upper rectum mass (2.9 cm), near-obstructing lesion
- 6 hepatic metastases (largest 6.3 cm), 1 pulmonary nodules (largest 0.9 cm), osseous metastases
- Other findings: Cholelithiasis, Hiatal hernia
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lung, liver, bone)
- Disease status: Newly diagnosed, treatment-naïve

- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 32053379

Date of Visit: June 15, 2025

Oncologist: Dr. David Thompson, MD

Chief Complaint

- Vomiting, Nausea, Weight loss

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 01, 2025
- Site of primary tumor: Transverse colon (right colon)
- Date of initial diagnosis and TNM stage: May 01, 2025; T3N0M1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 15, 2025 - lymph nodes, lung, bone metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Vomiting, Nausea, Weight loss, Constipation, Abdominal pain
- Performance status: ECOG 0
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Chronic kidney disease stage 2-3
 - Fibromyalgia
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Paternal uncle with prostate cancer
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Current smoker (10 pack-years)
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Nurse
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness

- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Multivitamin daily
- Allergies: Sulfa drugs (GI upset)

Physical Examination

- Vitals: BP 126/71, HR 75, RR 18, Temp 98.1°F, SpO2 97% on room air, Weight 148 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $4.5 \times 10^3/\mu\text{L}$, ANC: $3.4 \times 10^3/\mu\text{L}$
- Hgb: 12.7 g/dL, Platelets: $115 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.7 mg/dL, eGFR: 96 mL/min/1.73m²
- LFTs:
- AST: 85 U/L, ALT: 105 U/L
- Alkaline phosphatase: 221 U/L, Total bilirubin: 2.8 mg/dL
- CEA: 25.4 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G13D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 15, 2025):
- Primary transverse colon mass (6.8 cm), infiltrative mass
- 1 pulmonary nodules (largest 3.6 cm), enlarged lymph nodes (largest 3.8 cm), osseous metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lymph nodes, lung, bone)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 35523100

Date of Visit: June 14, 2025

Oncologist: Dr. Lisa Park, MD

Chief Complaint

- Bloating, Change in bowel habits, Fatigue

History of Present Illness

- Date of initial colorectal cancer diagnosis: June 01, 2025
- Site of primary tumor: Lower rectum (rectum)
- Date of initial diagnosis and TNM stage: June 01, 2025; T4bN2bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 14, 2025 - peritoneum metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Bloating, Change in bowel habits, Fatigue, Vomiting, Rectal bleeding
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Anxiety disorder
 - Inflammatory bowel disease
 - ADHD
- Surgical history:
 - Tubal ligation (2003)
 - Skin lesion removal (2008)
 - Hysterectomy (1990)

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: No known familial syndromes

Social History

- Smoking: Former smoker, quit 10 years ago (20 pack-year history)
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Office administrator
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing

- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Shellfish (mild GI upset)

Physical Examination

- Vitals: BP 133/90, HR 98, RR 15, Temp 99.0°F, SpO2 97% on room air, Weight 142 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $4.8 \times 10^3/\mu\text{L}$, ANC: $4.8 \times 10^3/\mu\text{L}$
- Hgb: 13.0 g/dL, Platelets: $282 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.0 mg/dL, eGFR: 112 mL/min/1.73m²
- LFTs:
- AST: 17 U/L, ALT: 22 U/L
- Alkaline phosphatase: 97 U/L, Total bilirubin: 0.7 mg/dL
- CEA: 48.3 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: V600E
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 14, 2025):
- Primary lower rectum mass (3.6 cm), near-obstructing lesion
- peritoneal disease
- Other findings: Atherosclerotic calcifications, Hepatic steatosis, Cholelithiasis
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (peritoneum)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time

- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 71696200

Date of Visit: June 11, 2025

Oncologist: Dr. Michael Chen, MD

Chief Complaint

- Fatigue, Nausea, Weight loss

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 18, 2025
- Site of primary tumor: Lower rectum (rectum)
- Date of initial diagnosis and TNM stage: May 18, 2025; T4aN1bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 11, 2025 - peritoneum metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Fatigue, Nausea, Weight loss
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Psoriasis
 - Hypertension
- Surgical history:
 - Tubal ligation (2019)
 - Appendectomy (2020)
 - Hernia repair (1993)
 - Hysterectomy (2020)

Family History

- Colorectal or other cancers in first-degree relatives: Paternal uncle with prostate cancer
- Known familial syndromes: No known familial syndromes

Social History

- Smoking: Former smoker, quit 5 years ago (15 pack-year history)
- Alcohol use: Social drinking, 2-3 drinks per week
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired teacher
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing

- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Amlodipine 5mg daily
- Multivitamin daily
- Allergies: Sulfa drugs (GI upset)

Physical Examination

- Vitals: BP 139/81, HR 89, RR 17, Temp 98.8°F, SpO2 100% on room air, Weight 160 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $5.7 \times 10^3/\mu\text{L}$, ANC: $7.4 \times 10^3/\mu\text{L}$
- Hgb: 13.6 g/dL, Platelets: $202 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.8 mg/dL, eGFR: 60 mL/min/1.73m²
- LFTs:
- AST: 15 U/L, ALT: 18 U/L
- Alkaline phosphatase: 105 U/L, Total bilirubin: 0.3 mg/dL
- CEA: 47.5 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G13D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 11, 2025):
- Primary lower rectum mass (6.9 cm), circumferential thickening peritoneal disease
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (peritoneum)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time

- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 18835665

Date of Visit: May 22, 2025

Oncologist: Dr. David Thompson, MD

Chief Complaint

- Abdominal pain, Constipation, Diarrhea

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 15, 2025
- Site of primary tumor: Lower rectum (rectum)
- Date of initial diagnosis and TNM stage: May 15, 2025; T4bN1aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 05, 2025 - brain, peritoneum, liver metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Abdominal pain, Constipation, Diarrhea, Bloating
- Performance status: ECOG 0
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Chronic kidney disease
 - Fibromyalgia
- Surgical history:
 - Prostate biopsy (2020)
 - Hernia repair (2022)
 - Testicular surgery (2011)

Family History

- Colorectal or other cancers in first-degree relatives: Mother with breast cancer at age 72
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Former smoker, quit 5 years ago (15 pack-year history)
- Alcohol use: Former alcohol use, quit 5 years ago
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired mechanic
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency

- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Multivitamin daily
- Allergies: NKDA (No Known Drug Allergies)

Physical Examination

- Vitals: BP 131/74, HR 95, RR 14, Temp 99.5°F, SpO2 100% on room air, Weight 146 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $5.7 \times 10^3/\mu\text{L}$, ANC: $3.5 \times 10^3/\mu\text{L}$
- Hgb: 12.2 g/dL, Platelets: $189 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.9 mg/dL, eGFR: 75 mL/min/1.73m²
- LFTs:
- AST: 27 U/L, ALT: 11 U/L
- Alkaline phosphatase: 44 U/L, Total bilirubin: 0.7 mg/dL
- CEA: 73.6 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G13D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 05, 2025):
- Primary lower rectum mass (6.7 cm), infiltrative mass
- 3 hepatic metastases (largest 6.4 cm), peritoneal disease, 3 brain metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (brain, peritoneum, liver)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 53972854

Date of Visit: June 05, 2025

Oncologist: Dr. Sarah Mitchell, MD

Chief Complaint

- Early satiety, Constipation, Nausea

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 12, 2025
- Site of primary tumor: Cecum (right colon)
- Date of initial diagnosis and TNM stage: April 12, 2025; T3N0M1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 09, 2025 - peritoneum, liver, lung metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Early satiety, Constipation, Nausea, Bloating
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - PCOS
 - Fibromyalgia
 - Chronic back pain
- Surgical history:
 - Appendectomy (1994)
 - Tonsillectomy (1990)

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Never smoker
- Alcohol use: Social drinking, 2-3 drinks per week
- Living situation/support: Lives with spouse, strong family support
- Occupation: Accountant
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency

- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Multivitamin daily
- Allergies: Penicillin (rash)

Physical Examination

- Vitals: BP 138/71, HR 75, RR 18, Temp 98.4°F, SpO2 96% on room air, Weight 180 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $10.1 \times 10^3/\mu\text{L}$, ANC: $7.3 \times 10^3/\mu\text{L}$
- Hgb: 14.6 g/dL, Platelets: $178 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.7 mg/dL, eGFR: 87 mL/min/1.73m²
- LFTs:
- AST: 38 U/L, ALT: 15 U/L
- Alkaline phosphatase: 137 U/L, Total bilirubin: 0.4 mg/dL
- CEA: 58.4 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSI-H

- Imaging:

- CT chest/abdomen/pelvis (June 09, 2025):
- Primary cecum mass (5.9 cm), near-obstructing lesion
- 6 hepatic metastases (largest 6.2 cm), 5 pulmonary nodules (largest 3.0 cm), peritoneal disease
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (peritoneum, liver, lung)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 57795494

Date of Visit: May 27, 2025

Oncologist: Dr. Sarah Mitchell, MD

Chief Complaint

- Bloating, Change in bowel habits, Cramping

History of Present Illness

- Date of initial colorectal cancer diagnosis: March 28, 2025
- Site of primary tumor: Descending colon (left colon)
- Date of initial diagnosis and TNM stage: March 28, 2025; T4bN1bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 13, 2025 - liver, peritoneum metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Bloating, Change in bowel habits, Cramping
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Chronic back pain
 - Anxiety disorder
 - Thyroid disorder
 - Migraine headaches
- Surgical history:
 - Tubal ligation (2021)
 - Ovarian cyst removal (2018)

Family History

- Colorectal or other cancers in first-degree relatives: Paternal uncle with prostate cancer
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Never smoker
- Alcohol use: Former alcohol use, quit 5 years ago
- Living situation/support: Lives with spouse, strong family support
- Occupation: Small business owner
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing

- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Multivitamin daily
- Allergies: Penicillin (rash)

Physical Examination

- Vitals: BP 138/80, HR 79, RR 17, Temp 98.0°F, SpO2 98% on room air, Weight 192 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $5.1 \times 10^3/\mu\text{L}$, ANC: $4.7 \times 10^3/\mu\text{L}$
- Hgb: 13.1 g/dL, Platelets: $221 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.1 mg/dL, eGFR: 116 mL/min/1.73m²
- LFTs:
- AST: 38 U/L, ALT: 34 U/L
- Alkaline phosphatase: 91 U/L, Total bilirubin: 0.4 mg/dL
- CEA: 121.4 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G12D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 13, 2025):
- Primary descending colon mass (5.8 cm), infiltrative mass
- 7 hepatic metastases (largest 5.9 cm), peritoneal disease
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (liver, peritoneum)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 99799928

Date of Visit: May 20, 2025

Oncologist: Dr. Sarah Mitchell, MD

Chief Complaint

- Rectal bleeding, Constipation, Nausea

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 24, 2025
- Site of primary tumor: Lower rectum (rectum)
- Date of initial diagnosis and TNM stage: April 24, 2025; T4bN1aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 06, 2025 - bone, liver metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Rectal bleeding, Constipation, Nausea, Vomiting
- Performance status: ECOG 0
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Macular degeneration
 - Chronic kidney disease stage 2-3
 - Chronic back pain
 - Hypertension
 - Type 2 diabetes
- Surgical history:
 - Tubal ligation (2018)
 - Cataract surgery (2020)
 - Cesarean section (1992)

Family History

- Colorectal or other cancers in first-degree relatives: Mother with breast cancer at age 72
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Former smoker, quit 10 years ago (20 pack-year history)
- Alcohol use: Rare alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Small business owner
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety

- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Amlodipine 5mg daily
- Metformin 1000mg twice daily
- Multivitamin daily
- Allergies: NKDA (No Known Drug Allergies)

Physical Examination

- Vitals: BP 119/87, HR 100, RR 17, Temp 99.1°F, SpO2 99% on room air, Weight 159 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $8.4 \times 10^3/\mu\text{L}$, ANC: $4.2 \times 10^3/\mu\text{L}$
- Hgb: 15.5 g/dL, Platelets: $135 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.4 mg/dL, eGFR: 98 mL/min/1.73m²
- LFTs:
- AST: 65 U/L, ALT: 107 U/L
- Alkaline phosphatase: 50 U/L, Total bilirubin: 0.8 mg/dL
- CEA: 22.5 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: V600E
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 06, 2025):
- Primary lower rectum mass (4.5 cm), circumferential thickening
- 8 hepatic metastases (largest 6.0 cm), osseous metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (bone, liver)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 78006419

Date of Visit: June 10, 2025

Oncologist: Dr. Jennifer Rodriguez, MD

Chief Complaint

- Constipation, Cramping, Vomiting

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 15, 2025
- Site of primary tumor: Lower rectum (rectum)
- Date of initial diagnosis and TNM stage: May 15, 2025; T2N2aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 15, 2025 - brain, bone, lung metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Constipation, Cramping, Vomiting
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Endometriosis
 - PCOS
 - Inflammatory bowel disease
- Surgical history:
 - Hernia repair (2020)
 - Tonsillectomy (1998)
 - Cesarean section (1991)
 - Hysterectomy (2012)

Family History

- Colorectal or other cancers in first-degree relatives: Father with colon cancer at age 68
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Former smoker, quit 5 years ago (15 pack-year history)
- Alcohol use: Former alcohol use, quit 5 years ago
- Living situation/support: Lives with spouse, strong family support
- Occupation: Sales manager
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath

- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Shellfish (mild GI upset)

Physical Examination

- Vitals: BP 116/73, HR 74, RR 17, Temp 98.1°F, SpO2 97% on room air, Weight 144 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $8.3 \times 10^3/\mu\text{L}$, ANC: $8.4 \times 10^3/\mu\text{L}$
- Hgb: 10.0 g/dL, Platelets: $220 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 2.0 mg/dL, eGFR: 108 mL/min/1.73m²
- LFTs:
- AST: 19 U/L, ALT: 87 U/L
- Alkaline phosphatase: 175 U/L, Total bilirubin: 0.6 mg/dL
- CEA: 199.6 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G13D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 15, 2025):
- Primary lower rectum mass (6.2 cm), circumferential thickening
- 3 pulmonary nodules (largest 3.2 cm), osseous metastases, 3 brain metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (brain, bone, lung)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time

- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 15887395

Date of Visit: May 19, 2025

Oncologist: Dr. Jennifer Rodriguez, MD

Chief Complaint

- Bloating, Cramping, Vomiting

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 16, 2025
- Site of primary tumor: Mid rectum (rectum)
- Date of initial diagnosis and TNM stage: April 16, 2025; T3N1aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 08, 2025 - lymph nodes, lung, peritoneum metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Bloating, Cramping, Vomiting, Rectal bleeding, Change in bowel habits
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Type 2 diabetes
 - Chronic back pain
- Surgical history:
 - Ovarian cyst removal (1994)

Family History

- Colorectal or other cancers in first-degree relatives: Paternal uncle with prostate cancer
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Current smoker (10 pack-years)
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired mechanic
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness

- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Metformin 1000mg twice daily
- Multivitamin daily
- Allergies: NKDA (No Known Drug Allergies)

Physical Examination

- Vitals: BP 112/82, HR 99, RR 14, Temp 98.6°F, SpO2 99% on room air, Weight 132 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $4.8 \times 10^3/\mu\text{L}$, ANC: $3.7 \times 10^3/\mu\text{L}$
- Hgb: 14.0 g/dL, Platelets: $183 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.0 mg/dL, eGFR: 112 mL/min/1.73m²
- LFTs:
- AST: 33 U/L, ALT: 19 U/L
- Alkaline phosphatase: 59 U/L, Total bilirubin: 1.2 mg/dL
- CEA: 165.9 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G12D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 08, 2025):
- Primary mid rectum mass (6.7 cm), polypoid mass
- 6 pulmonary nodules (largest 3.7 cm), peritoneal disease, enlarged lymph nodes (largest 3.5 cm)
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lymph nodes, lung, peritoneum)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 53280812

Date of Visit: June 10, 2025

Oncologist: Dr. Sarah Mitchell, MD

Chief Complaint

- Change in bowel habits, Nausea, Constipation

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 03, 2025
- Site of primary tumor: Sigmoid colon (left colon)
- Date of initial diagnosis and TNM stage: May 03, 2025; T2N1bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 09, 2025 - bone metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Change in bowel habits, Nausea, Constipation, Bloating, Weight loss
- Performance status: ECOG 0
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Type 2 diabetes
 - Psoriasis
 - Atrial fibrillation
 - Stroke history
 - Fibromyalgia
 - Osteoporosis
- Surgical history:
 - Cataract surgery (2000)
 - Circumcision (2023)
 - Vasectomy (2020)
 - Carpal tunnel release (2021)
 - Arthroscopy (2009)

Family History

- Colorectal or other cancers in first-degree relatives: Father with colon cancer at age 68
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Former smoker, quit 10 years ago (20 pack-year history)
- Alcohol use: Rare alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Small business owner
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Glipizide 5mg daily
- Multivitamin daily
- Allergies: Shellfish (mild GI upset)

Physical Examination

- Vitals: BP 135/85, HR 86, RR 16, Temp 98.2°F, SpO2 98% on room air, Weight 170 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $6.6 \times 10^3/\mu\text{L}$, ANC: $3.6 \times 10^3/\mu\text{L}$
- Hgb: 13.2 g/dL, Platelets: $198 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.9 mg/dL, eGFR: 73 mL/min/1.73m²
- LFTs:
- AST: 15 U/L, ALT: 15 U/L
- Alkaline phosphatase: 141 U/L, Total bilirubin: 0.9 mg/dL
- CEA: 144.1 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G12D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 09, 2025):
- Primary sigmoid colon mass (3.9 cm), near-obstructing lesion
- osseous metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (bone)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 95231752

Date of Visit: June 10, 2025

Oncologist: Dr. Michael Chen, MD

Chief Complaint

- Fatigue, Weight loss, Vomiting

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 14, 2025
- Site of primary tumor: Ascending colon (right colon)
- Date of initial diagnosis and TNM stage: April 14, 2025; T4aN2aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 04, 2025 - brain, lymph nodes, liver metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Fatigue, Weight loss, Vomiting, Early satiety, Diarrhea
- Performance status: ECOG 0
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Hypothyroidism
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Mother with breast cancer at age 72
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Former smoker, quit 5 years ago (15 pack-year history)
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired mechanic
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes

- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: NKDA (No Known Drug Allergies)

Physical Examination

- Vitals: BP 123/74, HR 78, RR 18, Temp 98.6°F, SpO2 99% on room air, Weight 160 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $4.3 \times 10^3/\mu\text{L}$, ANC: $5.7 \times 10^3/\mu\text{L}$
- Hgb: 15.2 g/dL, Platelets: $410 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.0 mg/dL, eGFR: 113 mL/min/1.73m²
- LFTs:
- AST: 20 U/L, ALT: 22 U/L
- Alkaline phosphatase: 93 U/L, Total bilirubin: 0.3 mg/dL
- CEA: 154.5 ng/mL (elevated)
- Molecular markers:
- RAS: NRAS Q61H
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 04, 2025):
- Primary ascending colon mass (5.2 cm), near-obstructing lesion
- 8 hepatic metastases (largest 6.3 cm), enlarged lymph nodes (largest 3.0 cm), 3 brain metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (brain, lymph nodes, liver)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 78352483

Date of Visit: June 01, 2025

Oncologist: Dr. Jennifer Rodriguez, MD

Chief Complaint

- Vomiting, Rectal bleeding, Change in bowel habits

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 08, 2025
- Site of primary tumor: Hepatic flexure (right colon)
- Date of initial diagnosis and TNM stage: April 08, 2025; T3N1aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 11, 2025 - lymph nodes, lung, liver metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Vomiting, Rectal bleeding, Change in bowel habits
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Hypothyroidism
 - Osteoarthritis
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Father with colon cancer at age 68
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Former smoker, quit 10 years ago (20 pack-year history)
- Alcohol use: Social drinking, 2-3 drinks per week
- Living situation/support: Lives with spouse, strong family support
- Occupation: Accountant
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness

- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Multivitamin daily
- Allergies: Penicillin (rash)

Physical Examination

- Vitals: BP 137/71, HR 92, RR 15, Temp 99.4°F, SpO2 100% on room air, Weight 175 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $9.4 \times 10^3/\mu\text{L}$, ANC: $3.8 \times 10^3/\mu\text{L}$
- Hgb: 12.7 g/dL, Platelets: $214 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.2 mg/dL, eGFR: 117 mL/min/1.73m²
- LFTs:
- AST: 19 U/L, ALT: 19 U/L
- Alkaline phosphatase: 102 U/L, Total bilirubin: 1.0 mg/dL
- CEA: 114.7 ng/mL (elevated)
- Molecular markers:
- RAS: NRAS Q61H
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 11, 2025):
- Primary hepatic flexure mass (3.7 cm), infiltrative mass
- 8 hepatic metastases (largest 6.1 cm), 5 pulmonary nodules (largest 3.8 cm), enlarged lymph nodes (largest 1.3 cm)
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lymph nodes, lung, liver)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 19001527

Date of Visit: May 24, 2025

Oncologist: Dr. Jennifer Rodriguez, MD

Chief Complaint

- Nausea, Abdominal pain, Rectal bleeding

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 07, 2025
- Site of primary tumor: Hepatic flexure (right colon)
- Date of initial diagnosis and TNM stage: April 07, 2025; T4aN1aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 08, 2025 - lung metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Nausea, Abdominal pain, Rectal bleeding, Fatigue, Diarrhea
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Anxiety disorder
 - Hypothyroidism
- Surgical history:
 - Hernia repair (2011)
 - Cholecystectomy (1997)

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Former smoker, quit 5 years ago (15 pack-year history)
- Alcohol use: Rare alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired teacher
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms

- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Multivitamin daily
- Allergies: Shellfish (mild GI upset)

Physical Examination

- Vitals: BP 123/90, HR 88, RR 20, Temp 98.3°F, SpO2 96% on room air, Weight 198 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $8.4 \times 10^3/\mu\text{L}$, ANC: $4.4 \times 10^3/\mu\text{L}$
- Hgb: 15.6 g/dL, Platelets: $309 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.9 mg/dL, eGFR: 108 mL/min/1.73m²
- LFTs:
- AST: 27 U/L, ALT: 16 U/L
- Alkaline phosphatase: 99 U/L, Total bilirubin: 0.8 mg/dL
- CEA: 145.9 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 08, 2025):
- Primary hepatic flexure mass (5.3 cm), annular narrowing
- 4 pulmonary nodules (largest 4.0 cm)
- Other findings: Renal cysts, Degenerative spine changes
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lung)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 40611637

Date of Visit: June 04, 2025

Oncologist: Dr. David Thompson, MD

Chief Complaint

- Cramping, Fatigue, Weight loss

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 25, 2025
- Site of primary tumor: Ascending colon (right colon)
- Date of initial diagnosis and TNM stage: April 25, 2025; T2N1bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 12, 2025 - bone, brain metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Cramping, Fatigue, Weight loss, Bloating
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Type 1 diabetes
 - Thyroid disorder
 - Chronic kidney disease stage 2-3
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Paternal uncle with prostate cancer
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Former smoker, quit 10 years ago (20 pack-year history)
- Alcohol use: Former alcohol use, quit 5 years ago
- Living situation/support: Lives with spouse, strong family support
- Occupation: Nurse
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms

- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Multivitamin daily
- Allergies: Shellfish (mild GI upset)

Physical Examination

- Vitals: BP 124/88, HR 81, RR 20, Temp 98.8°F, SpO2 99% on room air, Weight 165 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $12.4 \times 10^3/\mu\text{L}$, ANC: $11.6 \times 10^3/\mu\text{L}$
- Hgb: 14.2 g/dL, Platelets: $250 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.8 mg/dL, eGFR: 103 mL/min/1.73m²
- LFTs:
- AST: 48 U/L, ALT: 39 U/L
- Alkaline phosphatase: 107 U/L, Total bilirubin: 0.5 mg/dL
- CEA: 25.0 ng/mL (elevated)
- Molecular markers:
- RAS: NRAS Q61H
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 12, 2025):
- Primary ascending colon mass (6.7 cm), infiltrative mass
- osseous metastases, 1 brain metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (bone, brain)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 94833643

Date of Visit: May 25, 2025

Oncologist: Dr. Michael Chen, MD

Chief Complaint

- Early satiety, Constipation, Change in bowel habits

History of Present Illness

- Date of initial colorectal cancer diagnosis: March 29, 2025
- Site of primary tumor: Transverse colon (right colon)
- Date of initial diagnosis and TNM stage: March 29, 2025; T2N0M1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 10, 2025 - lung, brain, bone metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Early satiety, Constipation, Change in bowel habits, Weight loss, Nausea
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Osteoporosis
 - Coronary artery disease
 - Hyperlipidemia
 - Chronic back pain
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Former smoker, quit 10 years ago (20 pack-year history)
- Alcohol use: Rare alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired teacher
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency

- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Atorvastatin 20mg daily
- Multivitamin daily
- Allergies: Sulfa drugs (GI upset)

Physical Examination

- Vitals: BP 119/81, HR 100, RR 19, Temp 98.1°F, SpO2 97% on room air, Weight 144 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $7.5 \times 10^3/\mu\text{L}$, ANC: $4.3 \times 10^3/\mu\text{L}$
- Hgb: 15.1 g/dL, Platelets: $408 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.1 mg/dL, eGFR: 92 mL/min/1.73m²
- LFTs:
- AST: 22 U/L, ALT: 12 U/L
- Alkaline phosphatase: 50 U/L, Total bilirubin: 1.1 mg/dL
- CEA: 49.3 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G12D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 10, 2025):
- Primary transverse colon mass (5.6 cm), polypoid mass
- 2 pulmonary nodules (largest 3.2 cm), osseous metastases, 4 brain metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lung, brain, bone)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 37654454

Date of Visit: May 20, 2025

Oncologist: Dr. Lisa Park, MD

Chief Complaint

- Weight loss, Diarrhea, Nausea

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 13, 2025
- Site of primary tumor: Splenic flexure (left colon)
- Date of initial diagnosis and TNM stage: April 13, 2025; T4bN2bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 04, 2025 - peritoneum, brain metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Weight loss, Diarrhea, Nausea, Change in bowel habits, Vomiting, Cramping
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Type 1 diabetes
 - ADHD
 - Anxiety disorder
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Mother with breast cancer at age 72
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Former smoker, quit 10 years ago (20 pack-year history)
- Alcohol use: Social drinking, 2-3 drinks per week
- Living situation/support: Lives with spouse, strong family support
- Occupation: Sales manager
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms

- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: NKDA (No Known Drug Allergies)

Physical Examination

- Vitals: BP 129/73, HR 73, RR 15, Temp 99.2°F, SpO2 97% on room air, Weight 197 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $12.2 \times 10^3/\mu\text{L}$, ANC: $3.8 \times 10^3/\mu\text{L}$
- Hgb: 13.4 g/dL, Platelets: $305 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.1 mg/dL, eGFR: 53 mL/min/1.73m²
- LFTs:
- AST: 79 U/L, ALT: 64 U/L
- Alkaline phosphatase: 217 U/L, Total bilirubin: 0.3 mg/dL
- CEA: 88.4 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: V600E
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 04, 2025):
- Primary splenic flexure mass (3.2 cm), circumferential thickening peritoneal disease, 1 brain metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (peritoneum, brain)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 28618707

Date of Visit: May 28, 2025

Oncologist: Dr. Michael Chen, MD

Chief Complaint

- Weight loss, Cramping, Fatigue

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 15, 2025
- Site of primary tumor: Sigmoid colon (left colon)
- Date of initial diagnosis and TNM stage: April 15, 2025; T2N2aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 11, 2025 - lung, liver, lymph nodes metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Weight loss, Cramping, Fatigue, Change in bowel habits
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - None
- Surgical history:
 - Testicular surgery (1994)
 - Circumcision (1997)

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Never smoker
- Alcohol use: Former alcohol use, quit 5 years ago
- Living situation/support: Lives with spouse, strong family support
- Occupation: Sales manager
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness

- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: NKDA (No Known Drug Allergies)

Physical Examination

- Vitals: BP 116/71, HR 84, RR 17, Temp 98.2°F, SpO2 97% on room air, Weight 143 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $6.6 \times 10^3/\mu\text{L}$, ANC: $4.0 \times 10^3/\mu\text{L}$
- Hgb: 14.6 g/dL, Platelets: $302 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.8 mg/dL, eGFR: 82 mL/min/1.73m²
- LFTs:
- AST: 37 U/L, ALT: 15 U/L
- Alkaline phosphatase: 70 U/L, Total bilirubin: 1.2 mg/dL
- CEA: 25.1 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: V600E
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 11, 2025):
- Primary sigmoid colon mass (6.1 cm), infiltrative mass
- 3 hepatic metastases (largest 3.7 cm), 5 pulmonary nodules (largest 3.7 cm), enlarged lymph nodes (largest 2.8 cm)
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lung, liver, lymph nodes)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 86474119

Date of Visit: May 19, 2025

Oncologist: Dr. Michael Chen, MD

Chief Complaint

- Abdominal pain, Rectal bleeding, Weight loss

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 03, 2025
- Site of primary tumor: Splenic flexure (left colon)
- Date of initial diagnosis and TNM stage: April 03, 2025; T3N1bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 16, 2025 - brain, peritoneum metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Abdominal pain, Rectal bleeding, Weight loss, Fatigue
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Chronic fatigue syndrome
 - Sleep apnea
- Surgical history:
 - Tonsillectomy (2009)
 - Carpal tunnel release (2009)

Family History

- Colorectal or other cancers in first-degree relatives: No known family history of cancer
- Known familial syndromes: No known familial syndromes

Social History

- Smoking: Current smoker (10 pack-years)
- Alcohol use: Former alcohol use, quit 5 years ago
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired teacher
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms

- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Penicillin (rash), Shellfish (hives)

Physical Examination

- Vitals: BP 133/79, HR 95, RR 18, Temp 98.6°F, SpO2 98% on room air, Weight 173 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $4.3 \times 10^3/\mu\text{L}$, ANC: $3.5 \times 10^3/\mu\text{L}$
- Hgb: 12.5 g/dL, Platelets: $215 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.8 mg/dL, eGFR: 81 mL/min/1.73m²
- LFTs:
- AST: 40 U/L, ALT: 15 U/L
- Alkaline phosphatase: 107 U/L, Total bilirubin: 0.4 mg/dL
- CEA: 138.7 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSI-H

- Imaging:

- CT chest/abdomen/pelvis (June 16, 2025):
- Primary splenic flexure mass (5.7 cm), near-obstructing lesion
- peritoneal disease, 4 brain metastases
- Other findings: Pulmonary nodules (likely benign), Hiatal hernia
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (brain, peritoneum)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 25935014

Date of Visit: May 22, 2025

Oncologist: Dr. Michael Chen, MD

Chief Complaint

- Nausea, Weight loss, Constipation

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 13, 2025
- Site of primary tumor: Descending colon (left colon)
- Date of initial diagnosis and TNM stage: May 13, 2025; T4bN0M1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 15, 2025 - lung, bone metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Nausea, Weight loss, Constipation, Early satiety, Abdominal pain
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Dementia (early stage)
 - Peripheral artery disease
 - Sleep apnea
 - Hypertension
 - Psoriasis
- Surgical history:
 - Prostate biopsy (2012)
 - Arthroscopy (1995)

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Never smoker
- Alcohol use: Former alcohol use, quit 5 years ago
- Living situation/support: Lives with spouse, strong family support
- Occupation: Office administrator
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath

- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Metoprolol 50mg twice daily
- Multivitamin daily
- Allergies: Penicillin (rash)

Physical Examination

- Vitals: BP 131/70, HR 70, RR 16, Temp 99.3°F, SpO2 97% on room air, Weight 160 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $10.6 \times 10^3/\mu\text{L}$, ANC: $5.5 \times 10^3/\mu\text{L}$
- Hgb: 15.8 g/dL, Platelets: $166 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.0 mg/dL, eGFR: 118 mL/min/1.73m²
- LFTs:
- AST: 28 U/L, ALT: 23 U/L
- Alkaline phosphatase: 87 U/L, Total bilirubin: 0.8 mg/dL
- CEA: 107.5 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSI-H

- Imaging:

- CT chest/abdomen/pelvis (June 15, 2025):
- Primary descending colon mass (3.8 cm), near-obstructing lesion
- 6 pulmonary nodules (largest 2.5 cm), osseous metastases
- Other findings: Hiatal hernia, Renal cysts, Hepatic steatosis
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lung, bone)

- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 47094555

Date of Visit: June 14, 2025

Oncologist: Dr. Jennifer Rodriguez, MD

Chief Complaint

- Constipation, Abdominal pain, Diarrhea

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 12, 2025
- Site of primary tumor: Splenic flexure (left colon)
- Date of initial diagnosis and TNM stage: May 12, 2025; T3N1bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 03, 2025 - lymph nodes metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Constipation, Abdominal pain, Diarrhea
- Performance status: ECOG 0
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Hypothyroidism
 - Fibromyalgia
 - Heart failure
 - Stroke history
- Surgical history:
 - Tonsillectomy (2015)

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: No known familial syndromes

Social History

- Smoking: Never smoker
- Alcohol use: Rare alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Engineer
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency

- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Multivitamin daily
- Allergies: Penicillin (rash)

Physical Examination

- Vitals: BP 138/87, HR 86, RR 18, Temp 98.4°F, SpO2 98% on room air, Weight 147 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $4.5 \times 10^3/\mu\text{L}$, ANC: $5.0 \times 10^3/\mu\text{L}$
- Hgb: 14.9 g/dL, Platelets: $191 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.1 mg/dL, eGFR: 69 mL/min/1.73m²
- LFTs:
- AST: 28 U/L, ALT: 22 U/L
- Alkaline phosphatase: 116 U/L, Total bilirubin: 0.5 mg/dL
- CEA: 120.6 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSI-H

- Imaging:

- CT chest/abdomen/pelvis (June 03, 2025):
- Primary splenic flexure mass (3.1 cm), polypoid mass
- enlarged lymph nodes (largest 3.3 cm)
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lymph nodes)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 33439724

Date of Visit: June 04, 2025

Oncologist: Dr. Lisa Park, MD

Chief Complaint

- Cramping, Constipation, Bloating

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 22, 2025
- Site of primary tumor: Mid rectum (rectum)
- Date of initial diagnosis and TNM stage: April 22, 2025; T4bN1aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 09, 2025 - lymph nodes metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Cramping, Constipation, Bloating
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Type 1 diabetes
 - Chronic back pain
 - Depression
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Paternal uncle with prostate cancer
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Former smoker, quit 10 years ago (20 pack-year history)
- Alcohol use: Rare alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired teacher
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms

- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Sulfa drugs (GI upset)

Physical Examination

- Vitals: BP 138/71, HR 86, RR 17, Temp 98.0°F, SpO2 98% on room air, Weight 182 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $3.8 \times 10^3/\mu\text{L}$, ANC: $2.0 \times 10^3/\mu\text{L}$
- Hgb: 12.7 g/dL, Platelets: $342 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.7 mg/dL, eGFR: 111 mL/min/1.73m²
- LFTs:
- AST: 40 U/L, ALT: 27 U/L
- Alkaline phosphatase: 139 U/L, Total bilirubin: 0.4 mg/dL
- CEA: 104.9 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G12D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 09, 2025):
- Primary mid rectum mass (3.8 cm), polypoid mass
- enlarged lymph nodes (largest 2.3 cm)
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lymph nodes)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 69326125

Date of Visit: May 21, 2025

Oncologist: Dr. Sarah Mitchell, MD

Chief Complaint

- Change in bowel habits, Fatigue, Diarrhea

History of Present Illness

- Date of initial colorectal cancer diagnosis: March 31, 2025
- Site of primary tumor: Splenic flexure (left colon)
- Date of initial diagnosis and TNM stage: March 31, 2025; T3N0M1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 16, 2025 - liver, lung, peritoneum metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Change in bowel habits, Fatigue, Diarrhea, Bloating
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - None
- Surgical history:
 - Skin lesion removal (2012)

Family History

- Colorectal or other cancers in first-degree relatives: Father with colon cancer at age 68
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Former smoker, quit 5 years ago (15 pack-year history)
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Sales manager
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes

- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: NKDA (No Known Drug Allergies)

Physical Examination

- Vitals: BP 118/83, HR 81, RR 14, Temp 98.0°F, SpO2 100% on room air, Weight 193 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $3.9 \times 10^3/\mu\text{L}$, ANC: $5.5 \times 10^3/\mu\text{L}$
- Hgb: 12.2 g/dL, Platelets: $280 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.0 mg/dL, eGFR: 66 mL/min/1.73m²
- LFTs:
- AST: 18 U/L, ALT: 33 U/L
- Alkaline phosphatase: 88 U/L, Total bilirubin: 0.4 mg/dL
- CEA: 182.6 ng/mL (elevated)
- Molecular markers:
- RAS: NRAS Q61H
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 16, 2025):
- Primary splenic flexure mass (5.8 cm), annular narrowing
- 8 hepatic metastases (largest 6.2 cm), 4 pulmonary nodules (largest 1.7 cm), peritoneal disease
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (liver, lung, peritoneum)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 97136901

Date of Visit: May 22, 2025

Oncologist: Dr. David Thompson, MD

Chief Complaint

- Early satiety, Nausea, Change in bowel habits

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 25, 2025
- Site of primary tumor: Splenic flexure (left colon)
- Date of initial diagnosis and TNM stage: April 25, 2025; T2N2bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 03, 2025 - peritoneum, liver metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Early satiety, Nausea, Change in bowel habits, Rectal bleeding, Fatigue, Cramping
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Endometriosis
 - Chronic back pain
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: No known family history of cancer
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Former smoker, quit 5 years ago (15 pack-year history)
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Small business owner
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms

- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Penicillin (rash)

Physical Examination

- Vitals: BP 125/87, HR 90, RR 20, Temp 99.1°F, SpO2 99% on room air, Weight 190 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $9.6 \times 10^3/\mu\text{L}$, ANC: $6.5 \times 10^3/\mu\text{L}$
- Hgb: 15.6 g/dL, Platelets: $428 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.1 mg/dL, eGFR: 91 mL/min/1.73m²
- LFTs:
- AST: 40 U/L, ALT: 35 U/L
- Alkaline phosphatase: 113 U/L, Total bilirubin: 1.1 mg/dL
- CEA: 41.6 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G13D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 03, 2025):
- Primary splenic flexure mass (6.6 cm), near-obstructing lesion
- 1 hepatic metastases (largest 2.3 cm), peritoneal disease
- Other findings: Hepatic steatosis, Hiatal hernia
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (peritoneum, liver)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 82074147

Date of Visit: June 06, 2025

Oncologist: Dr. Lisa Park, MD

Chief Complaint

- Nausea, Abdominal pain, Diarrhea

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 24, 2025
- Site of primary tumor: Mid rectum (rectum)
- Date of initial diagnosis and TNM stage: May 24, 2025; T3N1bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 10, 2025 - lymph nodes metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Nausea, Abdominal pain, Diarrhea, Rectal bleeding, Fatigue, Bloating
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - None
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: No known family history of cancer
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Former smoker, quit 5 years ago (15 pack-year history)
- Alcohol use: Rare alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Small business owner
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes

- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Sulfa drugs (GI upset)

Physical Examination

- Vitals: BP 132/77, HR 72, RR 17, Temp 99.2°F, SpO2 97% on room air, Weight 141 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $11.7 \times 10^3/\mu\text{L}$, ANC: $6.9 \times 10^3/\mu\text{L}$
- Hgb: 14.5 g/dL, Platelets: $342 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.7 mg/dL, eGFR: 103 mL/min/1.73m²
- LFTs:
- AST: 18 U/L, ALT: 16 U/L
- Alkaline phosphatase: 56 U/L, Total bilirubin: 0.5 mg/dL
- CEA: 94.2 ng/mL (elevated)
- Molecular markers:
- RAS: NRAS Q61H
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 10, 2025):
Primary mid rectum mass (7.3 cm), near-obstructing lesion
enlarged lymph nodes (largest 2.1 cm)
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lymph nodes)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 57391738

Date of Visit: May 24, 2025

Oncologist: Dr. David Thompson, MD

Chief Complaint

- Cramping, Diarrhea, Change in bowel habits

History of Present Illness

- Date of initial colorectal cancer diagnosis: March 26, 2025
- Site of primary tumor: Upper rectum (rectum)
- Date of initial diagnosis and TNM stage: March 26, 2025; T4aN0M1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 15, 2025 - brain, lymph nodes metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Cramping, Diarrhea, Change in bowel habits, Vomiting
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Migraine headaches
 - Thyroid disorder
 - Endometriosis
- Surgical history:
 - Hernia repair (2021)
 - D&C; (1993)
 - Cesarean section (1992)

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Never smoker
- Alcohol use: Former alcohol use, quit 5 years ago
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired mechanic
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing

- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: NKDA (No Known Drug Allergies)

Physical Examination

- Vitals: BP 110/85, HR 72, RR 17, Temp 98.5°F, SpO2 100% on room air, Weight 133 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $9.6 \times 10^3/\mu\text{L}$, ANC: $2.4 \times 10^3/\mu\text{L}$
- Hgb: 14.7 g/dL, Platelets: $415 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.0 mg/dL, eGFR: 77 mL/min/1.73m²
- LFTs:
- AST: 39 U/L, ALT: 28 U/L
- Alkaline phosphatase: 75 U/L, Total bilirubin: 0.2 mg/dL
- CEA: 95.3 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSI-H

- Imaging:

- CT chest/abdomen/pelvis (June 15, 2025):
- Primary upper rectum mass (5.2 cm), infiltrative mass
- enlarged lymph nodes (largest 2.0 cm), 4 brain metastases
- Other findings: Degenerative spine changes, Diverticulosis
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (brain, lymph nodes)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time

- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 63472383

Date of Visit: June 10, 2025

Oncologist: Dr. David Thompson, MD

Chief Complaint

- Bloating, Rectal bleeding, Diarrhea

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 26, 2025
- Site of primary tumor: Cecum (right colon)
- Date of initial diagnosis and TNM stage: May 26, 2025; T2N2bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 16, 2025 - lung, liver, peritoneum metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Bloating, Rectal bleeding, Diarrhea, Early satiety, Weight loss
- Performance status: ECOG 0
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
- Inflammatory bowel disease
- Surgical history:
- None

Family History

- Colorectal or other cancers in first-degree relatives: Paternal uncle with prostate cancer
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Former smoker, quit 10 years ago (20 pack-year history)
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Small business owner
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes

- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Sulfa drugs (GI upset)

Physical Examination

- Vitals: BP 119/80, HR 93, RR 16, Temp 98.6°F, SpO2 97% on room air, Weight 149 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $11.2 \times 10^3/\mu\text{L}$, ANC: $5.4 \times 10^3/\mu\text{L}$
- Hgb: 13.6 g/dL, Platelets: $241 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.8 mg/dL, eGFR: 73 mL/min/1.73m²
- LFTs:
- AST: 18 U/L, ALT: 18 U/L
- Alkaline phosphatase: 118 U/L, Total bilirubin: 1.0 mg/dL
- CEA: 190.6 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: V600E
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 16, 2025):
- Primary cecum mass (7.3 cm), circumferential thickening
- 6 hepatic metastases (largest 6.2 cm), 1 pulmonary nodules (largest 3.4 cm), peritoneal disease
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lung, liver, peritoneum)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 58918645

Date of Visit: May 25, 2025

Oncologist: Dr. Lisa Park, MD

Chief Complaint

- Vomiting, Cramping, Early satiety

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 14, 2025
- Site of primary tumor: Mid rectum (rectum)
- Date of initial diagnosis and TNM stage: May 14, 2025; T4aN0M1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 08, 2025 - bone, liver metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Vomiting, Cramping, Early satiety, Diarrhea, Rectal bleeding
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Atrial fibrillation
 - Depression
 - Sleep apnea
 - Chronic kidney disease stage 2-3
 - Hypertension
- Surgical history:
 - Skin lesion removal (2019)

Family History

- Colorectal or other cancers in first-degree relatives: No known family history of cancer
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Former smoker, quit 5 years ago (15 pack-year history)
- Alcohol use: Rare alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired teacher
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing

- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Lisinopril 10mg daily
- Multivitamin daily
- Allergies: Sulfa drugs (GI upset)

Physical Examination

- Vitals: BP 112/90, HR 72, RR 18, Temp 98.3°F, SpO2 100% on room air, Weight 182 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $8.8 \times 10^3/\mu\text{L}$, ANC: $2.6 \times 10^3/\mu\text{L}$
- Hgb: 12.8 g/dL, Platelets: $276 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.9 mg/dL, eGFR: 92 mL/min/1.73m²
- LFTs:
- AST: 82 U/L, ALT: 12 U/L
- Alkaline phosphatase: 49 U/L, Total bilirubin: 2.1 mg/dL
- CEA: 185.9 ng/mL (elevated)
- Molecular markers:
- RAS: NRAS Q61H
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 08, 2025):
- Primary mid rectum mass (3.5 cm), annular narrowing
- 7 hepatic metastases (largest 5.2 cm), osseous metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (bone, liver)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time

- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 43935511

Date of Visit: June 14, 2025

Oncologist: Dr. Michael Chen, MD

Chief Complaint

- Cramping, Weight loss, Vomiting

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 13, 2025
- Site of primary tumor: Upper rectum (rectum)
- Date of initial diagnosis and TNM stage: May 13, 2025; T2N1aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 12, 2025 - brain, peritoneum, bone metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Cramping, Weight loss, Vomiting, Constipation, Abdominal pain
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Osteoporosis
 - Depression
- Surgical history:
 - Breast biopsy (1992)
 - Hysterectomy (2022)

Family History

- Colorectal or other cancers in first-degree relatives: Paternal uncle with prostate cancer
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Never smoker
- Alcohol use: Former alcohol use, quit 5 years ago
- Living situation/support: Lives with spouse, strong family support
- Occupation: Small business owner
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms

- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Multivitamin daily
- Allergies: Sulfa drugs (GI upset)

Physical Examination

- Vitals: BP 139/74, HR 70, RR 15, Temp 99.5°F, SpO2 96% on room air, Weight 191 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $8.5 \times 10^3/\mu\text{L}$, ANC: $5.6 \times 10^3/\mu\text{L}$
- Hgb: 9.4 g/dL, Platelets: $177 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.6 mg/dL, eGFR: 56 mL/min/1.73m²
- LFTs:
- AST: 25 U/L, ALT: 14 U/L
- Alkaline phosphatase: 226 U/L, Total bilirubin: 2.5 mg/dL
- CEA: 189.7 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 12, 2025):
- Primary upper rectum mass (3.9 cm), circumferential thickening
- peritoneal disease, osseous metastases, 3 brain metastases
- Other findings: Atherosclerotic calcifications, Degenerative spine changes
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (brain, peritoneum, bone)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 25665253

Date of Visit: May 24, 2025

Oncologist: Dr. Michael Chen, MD

Chief Complaint

- Fatigue, Early satiety, Change in bowel habits

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 07, 2025
- Site of primary tumor: Descending colon (left colon)
- Date of initial diagnosis and TNM stage: May 07, 2025; T4aN1aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 08, 2025 - bone, liver metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Fatigue, Early satiety, Change in bowel habits
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
- Chronic fatigue syndrome
- ADHD
- Surgical history:
- None

Family History

- Colorectal or other cancers in first-degree relatives: Father with colon cancer at age 68
- Known familial syndromes: No known familial syndromes

Social History

- Smoking: Former smoker, quit 10 years ago (20 pack-year history)
- Alcohol use: Social drinking, 2-3 drinks per week
- Living situation/support: Lives with spouse, strong family support
- Occupation: Office administrator
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness

- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Penicillin (rash)

Physical Examination

- Vitals: BP 133/82, HR 70, RR 14, Temp 99.0°F, SpO2 96% on room air, Weight 169 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $5.0 \times 10^3/\mu\text{L}$, ANC: $5.8 \times 10^3/\mu\text{L}$
- Hgb: 13.4 g/dL, Platelets: $210 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.8 mg/dL, eGFR: 118 mL/min/1.73m²
- LFTs:
- AST: 22 U/L, ALT: 32 U/L
- Alkaline phosphatase: 54 U/L, Total bilirubin: 0.9 mg/dL
- CEA: 163.6 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G12D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 08, 2025):
- Primary descending colon mass (5.2 cm), near-obstructing lesion
- 3 hepatic metastases (largest 5.4 cm), osseous metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (bone, liver)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 55525103

Date of Visit: May 31, 2025

Oncologist: Dr. Michael Chen, MD

Chief Complaint

- Nausea, Early satiety, Abdominal pain

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 24, 2025
- Site of primary tumor: Lower rectum (rectum)
- Date of initial diagnosis and TNM stage: April 24, 2025; T4bN1aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 14, 2025 - lung metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Nausea, Early satiety, Abdominal pain, Change in bowel habits, Cramping, Diarrhea
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - COPD
 - Stroke history
 - Peripheral artery disease
- Surgical history:
 - Cholecystectomy (1998)
 - Tonsillectomy (2015)
 - Pacemaker insertion (2009)

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: No known familial syndromes

Social History

- Smoking: Current smoker (10 pack-years)
- Alcohol use: Rare alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Sales manager
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing

- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Multivitamin daily
- Allergies: Penicillin (rash), Shellfish (hives)

Physical Examination

- Vitals: BP 122/71, HR 86, RR 19, Temp 98.3°F, SpO2 97% on room air, Weight 183 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $7.5 \times 10^3/\mu\text{L}$, ANC: $4.5 \times 10^3/\mu\text{L}$
- Hgb: 13.7 g/dL, Platelets: $169 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.8 mg/dL, eGFR: 110 mL/min/1.73m²
- LFTs:
- AST: 40 U/L, ALT: 30 U/L
- Alkaline phosphatase: 59 U/L, Total bilirubin: 0.4 mg/dL
- CEA: 167.9 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: V600E
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 14, 2025):
- Primary lower rectum mass (8.0 cm), infiltrative mass
- 6 pulmonary nodules (largest 4.0 cm)
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lung)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 85048858

Date of Visit: May 24, 2025

Oncologist: Dr. Sarah Mitchell, MD

Chief Complaint

- Fatigue, Cramping, Weight loss

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 14, 2025
- Site of primary tumor: Hepatic flexure (right colon)
- Date of initial diagnosis and TNM stage: May 14, 2025; T4aN0M1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 07, 2025 - brain metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Fatigue, Cramping, Weight loss, Vomiting, Constipation
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Depression
 - Celiac disease
 - Chronic fatigue syndrome
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Current smoker (10 pack-years)
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Accountant
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms

- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Penicillin (rash)

Physical Examination

- Vitals: BP 118/89, HR 91, RR 18, Temp 98.5°F, SpO2 96% on room air, Weight 182 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $3.6 \times 10^3/\mu\text{L}$, ANC: $7.3 \times 10^3/\mu\text{L}$
- Hgb: 8.9 g/dL, Platelets: $255 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.5 mg/dL, eGFR: 118 mL/min/1.73m²
- LFTs:
- AST: 86 U/L, ALT: 34 U/L
- Alkaline phosphatase: 74 U/L, Total bilirubin: 2.9 mg/dL
- CEA: 191.9 ng/mL (elevated)
- Molecular markers:
- RAS: NRAS Q61H
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 07, 2025):
- Primary hepatic flexure mass (4.5 cm), polypoid mass
- 1 brain metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (brain)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 97339820

Date of Visit: June 02, 2025

Oncologist: Dr. Lisa Park, MD

Chief Complaint

- Nausea, Fatigue, Constipation

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 24, 2025
- Site of primary tumor: Lower rectum (rectum)
- Date of initial diagnosis and TNM stage: May 24, 2025; T2N1aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 03, 2025 - liver, lymph nodes, bone metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Nausea, Fatigue, Constipation, Diarrhea, Bloating, Abdominal pain
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Chronic kidney disease
 - Dementia (early stage)
 - Depression
 - Benign prostatic hyperplasia
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Current smoker (10 pack-years)
- Alcohol use: Rare alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Accountant
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency

- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Multivitamin daily
- Allergies: Sulfa drugs (GI upset)

Physical Examination

- Vitals: BP 122/73, HR 94, RR 15, Temp 99.3°F, SpO2 100% on room air, Weight 184 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $10.7 \times 10^3/\mu\text{L}$, ANC: $2.6 \times 10^3/\mu\text{L}$
- Hgb: 9.2 g/dL, Platelets: $161 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.7 mg/dL, eGFR: 99 mL/min/1.73m²
- LFTs:
- AST: 26 U/L, ALT: 89 U/L
- Alkaline phosphatase: 109 U/L, Total bilirubin: 0.7 mg/dL
- CEA: 181.8 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSI-H

- Imaging:

- CT chest/abdomen/pelvis (June 03, 2025):
- Primary lower rectum mass (6.7 cm), annular narrowing
- 7 hepatic metastases (largest 4.2 cm), enlarged lymph nodes (largest 3.3 cm), osseous metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (liver, lymph nodes, bone)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 47922542

Date of Visit: June 13, 2025

Oncologist: Dr. Jennifer Rodriguez, MD

Chief Complaint

- Weight loss, Vomiting, Fatigue

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 17, 2025
- Site of primary tumor: Splenic flexure (left colon)
- Date of initial diagnosis and TNM stage: May 17, 2025; T2N2aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 08, 2025 - lymph nodes, brain, peritoneum metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Weight loss, Vomiting, Fatigue, Cramping, Early satiety, Diarrhea
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Hypertension
 - Type 2 diabetes
 - Hyperlipidemia
 - Psoriasis
 - Heart failure
 - Dementia (early stage)
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: No known family history of cancer
- Known familial syndromes: No known familial syndromes

Social History

- Smoking: Never smoker
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Nurse
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath

- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Lisinopril 10mg daily
- Glipizide 5mg daily
- Simvastatin 40mg daily
- Multivitamin daily
- Allergies: Sulfa drugs (GI upset)

Physical Examination

- Vitals: BP 127/75, HR 78, RR 19, Temp 98.4°F, SpO2 98% on room air, Weight 147 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $7.7 \times 10^3/\mu\text{L}$, ANC: $2.6 \times 10^3/\mu\text{L}$
- Hgb: 13.3 g/dL, Platelets: $386 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.5 mg/dL, eGFR: 55 mL/min/1.73m²
- LFTs:
- AST: 69 U/L, ALT: 65 U/L
- Alkaline phosphatase: 300 U/L, Total bilirubin: 2.0 mg/dL
- CEA: 16.3 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 08, 2025):
- Primary splenic flexure mass (5.1 cm), annular narrowing
- peritoneal disease, enlarged lymph nodes (largest 3.7 cm), 2 brain metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lymph nodes, brain, peritoneum)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 56838444

Date of Visit: June 06, 2025

Oncologist: Dr. Michael Chen, MD

Chief Complaint

- Rectal bleeding, Cramping, Constipation

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 23, 2025
- Site of primary tumor: Mid rectum (rectum)
- Date of initial diagnosis and TNM stage: April 23, 2025; T3N1bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 06, 2025 - lung, bone metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Rectal bleeding, Cramping, Constipation
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - GERD
 - Chronic back pain
 - Osteoarthritis
 - Macular degeneration
 - Type 2 diabetes
 - Stroke history
- Surgical history:
 - Vasectomy (2019)

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Former smoker, quit 5 years ago (15 pack-year history)
- Alcohol use: Social drinking, 2-3 drinks per week
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired teacher
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath

- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Metformin 1000mg twice daily
- Multivitamin daily
- Allergies: Shellfish (mild GI upset)

Physical Examination

- Vitals: BP 117/78, HR 86, RR 16, Temp 98.6°F, SpO2 99% on room air, Weight 166 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $9.6 \times 10^3/\mu\text{L}$, ANC: $9.2 \times 10^3/\mu\text{L}$
- Hgb: 16.0 g/dL, Platelets: $367 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.8 mg/dL, eGFR: 33 mL/min/1.73m²
- LFTs:
- AST: 56 U/L, ALT: 46 U/L
- Alkaline phosphatase: 58 U/L, Total bilirubin: 2.6 mg/dL
- CEA: 160.8 ng/mL (elevated)
- Molecular markers:
- RAS: NRAS Q61H
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 06, 2025):
- Primary mid rectum mass (3.5 cm), near-obstructing lesion
- 2 pulmonary nodules (largest 3.3 cm), osseous metastases
- Other findings: Renal cysts, Diverticulosis
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lung, bone)

- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 93949889

Date of Visit: June 05, 2025

Oncologist: Dr. Michael Chen, MD

Chief Complaint

- Vomiting, Diarrhea, Nausea

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 15, 2025
- Site of primary tumor: Sigmoid colon (left colon)
- Date of initial diagnosis and TNM stage: May 15, 2025; T2N2bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 05, 2025 - bone metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Vomiting, Diarrhea, Nausea
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - None
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Mother with breast cancer at age 72
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Former smoker, quit 10 years ago (20 pack-year history)
- Alcohol use: Former alcohol use, quit 5 years ago
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired teacher
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes

- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: NKDA (No Known Drug Allergies)

Physical Examination

- Vitals: BP 131/89, HR 100, RR 18, Temp 99.4°F, SpO2 97% on room air, Weight 183 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $11.6 \times 10^3/\mu\text{L}$, ANC: $7.4 \times 10^3/\mu\text{L}$
- Hgb: 13.7 g/dL, Platelets: $188 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.8 mg/dL, eGFR: 93 mL/min/1.73m²
- LFTs:
- AST: 27 U/L, ALT: 34 U/L
- Alkaline phosphatase: 75 U/L, Total bilirubin: 0.5 mg/dL
- CEA: 23.4 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G13D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 05, 2025):
Primary sigmoid colon mass (7.3 cm), near-obstructing lesion
osseous metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (bone)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 17894757

Date of Visit: June 05, 2025

Oncologist: Dr. Jennifer Rodriguez, MD

Chief Complaint

- Cramping, Nausea, Weight loss

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 24, 2025
- Site of primary tumor: Descending colon (left colon)
- Date of initial diagnosis and TNM stage: May 24, 2025; T4aN0M1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 16, 2025 - brain metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Cramping, Nausea, Weight loss, Change in bowel habits, Rectal bleeding, Bloating
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Inflammatory bowel disease
 - Anxiety disorder
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Paternal uncle with prostate cancer
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Current smoker (10 pack-years)
- Alcohol use: Rare alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired teacher
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness

- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Penicillin (rash)

Physical Examination

- Vitals: BP 133/81, HR 93, RR 17, Temp 99.0°F, SpO2 97% on room air, Weight 150 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $12.6 \times 10^3/\mu\text{L}$, ANC: $4.6 \times 10^3/\mu\text{L}$
- Hgb: 14.2 g/dL, Platelets: $140 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.6 mg/dL, eGFR: 85 mL/min/1.73m²
- LFTs:
- AST: 40 U/L, ALT: 41 U/L
- Alkaline phosphatase: 220 U/L, Total bilirubin: 0.3 mg/dL
- CEA: 135.6 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 16, 2025):
- Primary descending colon mass (7.7 cm), annular narrowing
- 4 brain metastases
- Other findings: Cholelithiasis
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (brain)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 60774091

Date of Visit: June 13, 2025

Oncologist: Dr. Lisa Park, MD

Chief Complaint

- Vomiting, Change in bowel habits, Cramping

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 23, 2025
- Site of primary tumor: Mid rectum (rectum)
- Date of initial diagnosis and TNM stage: May 23, 2025; T3N1bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 13, 2025 - peritoneum metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Vomiting, Change in bowel habits, Cramping, Nausea
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - None
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Never smoker
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Small business owner
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes

- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Penicillin (rash)

Physical Examination

- Vitals: BP 138/83, HR 90, RR 18, Temp 99.4°F, SpO2 97% on room air, Weight 123 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $11.8 \times 10^3/\mu\text{L}$, ANC: $3.9 \times 10^3/\mu\text{L}$
- Hgb: 14.7 g/dL, Platelets: $425 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.9 mg/dL, eGFR: 84 mL/min/1.73m²
- LFTs:
- AST: 28 U/L, ALT: 17 U/L
- Alkaline phosphatase: 95 U/L, Total bilirubin: 0.2 mg/dL
- CEA: 164.9 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G12D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 13, 2025):
- Primary mid rectum mass (6.0 cm), polypoid mass
- peritoneal disease
- Other findings: Hepatic steatosis, Cholelithiasis, Degenerative spine changes
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (peritoneum)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 30480460

Date of Visit: May 26, 2025

Oncologist: Dr. David Thompson, MD

Chief Complaint

- Nausea, Bloating, Constipation

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 07, 2025
- Site of primary tumor: Lower rectum (rectum)
- Date of initial diagnosis and TNM stage: May 07, 2025; T3N1aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 14, 2025 - lung, liver metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Nausea, Bloating, Constipation
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Chronic fatigue syndrome
 - Celiac disease
- Surgical history:
 - Carpal tunnel release (2023)

Family History

- Colorectal or other cancers in first-degree relatives: Mother with breast cancer at age 72
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Former smoker, quit 5 years ago (15 pack-year history)
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired mechanic
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness

- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Sulfa drugs (GI upset)

Physical Examination

- Vitals: BP 137/76, HR 92, RR 15, Temp 98.4°F, SpO2 99% on room air, Weight 145 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $7.4 \times 10^3/\mu\text{L}$, ANC: $4.5 \times 10^3/\mu\text{L}$
- Hgb: 12.7 g/dL, Platelets: $446 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.0 mg/dL, eGFR: 82 mL/min/1.73m²
- LFTs:
- AST: 73 U/L, ALT: 89 U/L
- Alkaline phosphatase: 112 U/L, Total bilirubin: 0.5 mg/dL
- CEA: 160.0 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 14, 2025):
- Primary lower rectum mass (6.5 cm), infiltrative mass
- 3 hepatic metastases (largest 3.8 cm), 1 pulmonary nodules (largest 1.7 cm)
- Other findings: Pulmonary nodules (likely benign)
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lung, liver)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 90280004

Date of Visit: June 03, 2025

Oncologist: Dr. David Thompson, MD

Chief Complaint

- Fatigue, Vomiting, Weight loss

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 27, 2025
- Site of primary tumor: Splenic flexure (left colon)
- Date of initial diagnosis and TNM stage: May 27, 2025; T4aN0M1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 03, 2025 - lung metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Fatigue, Vomiting, Weight loss
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Depression
 - Thyroid disorder
 - Asthma
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Current smoker (10 pack-years)
- Alcohol use: Former alcohol use, quit 5 years ago
- Living situation/support: Lives with spouse, strong family support
- Occupation: Nurse
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms

- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: NKDA (No Known Drug Allergies)

Physical Examination

- Vitals: BP 117/70, HR 80, RR 19, Temp 99.1°F, SpO2 96% on room air, Weight 129 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $7.6 \times 10^3/\mu\text{L}$, ANC: $2.2 \times 10^3/\mu\text{L}$
- Hgb: 12.4 g/dL, Platelets: $382 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.9 mg/dL, eGFR: 85 mL/min/1.73m²
- LFTs:
- AST: 32 U/L, ALT: 23 U/L
- Alkaline phosphatase: 130 U/L, Total bilirubin: 1.0 mg/dL
- CEA: 10.1 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G13D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 03, 2025):
- Primary splenic flexure mass (3.9 cm), polypoid mass
- 3 pulmonary nodules (largest 3.6 cm)
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lung)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 63210266

Date of Visit: June 03, 2025

Oncologist: Dr. Sarah Mitchell, MD

Chief Complaint

- Change in bowel habits, Weight loss, Rectal bleeding

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 26, 2025
- Site of primary tumor: Upper rectum (rectum)
- Date of initial diagnosis and TNM stage: May 26, 2025; T4aN0M1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 09, 2025 - bone metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Change in bowel habits, Weight loss, Rectal bleeding, Nausea, Cramping
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - None
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Mother with breast cancer at age 72
- Known familial syndromes: No known familial syndromes

Social History

- Smoking: Former smoker, quit 5 years ago (15 pack-year history)
- Alcohol use: Former alcohol use, quit 5 years ago
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired teacher
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes

- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Penicillin (rash), Shellfish (hives)

Physical Examination

- Vitals: BP 131/87, HR 71, RR 20, Temp 98.6°F, SpO2 97% on room air, Weight 169 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $11.0 \times 10^3/\mu\text{L}$, ANC: $11.5 \times 10^3/\mu\text{L}$
- Hgb: 14.5 g/dL, Platelets: $298 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.1 mg/dL, eGFR: 59 mL/min/1.73m²
- LFTs:
- AST: 94 U/L, ALT: 119 U/L
- Alkaline phosphatase: 208 U/L, Total bilirubin: 1.1 mg/dL
- CEA: 108.2 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSI-H

- Imaging:

- CT chest/abdomen/pelvis (June 09, 2025):
Primary upper rectum mass (7.9 cm), infiltrative mass
osseous metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (bone)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 65601747

Date of Visit: June 05, 2025

Oncologist: Dr. Jennifer Rodriguez, MD

Chief Complaint

- Vomiting, Rectal bleeding, Early satiety

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 20, 2025
- Site of primary tumor: Transverse colon (right colon)
- Date of initial diagnosis and TNM stage: May 20, 2025; T2N1bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 14, 2025 - lung, lymph nodes, brain metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Vomiting, Rectal bleeding, Early satiety
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Dementia (early stage)
 - Hypothyroidism
 - Type 2 diabetes
 - GERD
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Never smoker
- Alcohol use: Rare alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Sales manager
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency

- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Metformin 1000mg twice daily
- Multivitamin daily
- Allergies: NKDA (No Known Drug Allergies)

Physical Examination

- Vitals: BP 128/81, HR 82, RR 19, Temp 98.6°F, SpO2 96% on room air, Weight 143 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $9.7 \times 10^3/\mu\text{L}$, ANC: $3.2 \times 10^3/\mu\text{L}$
- Hgb: 10.2 g/dL, Platelets: $142 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.1 mg/dL, eGFR: 39 mL/min/1.73m²
- LFTs:
- AST: 84 U/L, ALT: 60 U/L
- Alkaline phosphatase: 103 U/L, Total bilirubin: 2.2 mg/dL
- CEA: 35.7 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G12D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 14, 2025):
- Primary transverse colon mass (6.1 cm), near-obstructing lesion
- 4 pulmonary nodules (largest 3.4 cm), enlarged lymph nodes (largest 1.7 cm), 1 brain metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lung, lymph nodes, brain)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 74332303

Date of Visit: May 28, 2025

Oncologist: Dr. David Thompson, MD

Chief Complaint

- Constipation, Bloating, Nausea

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 21, 2025
- Site of primary tumor: Sigmoid colon (left colon)
- Date of initial diagnosis and TNM stage: April 21, 2025; T2N2aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 04, 2025 - liver, lymph nodes, brain metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Constipation, Bloating, Nausea, Rectal bleeding, Vomiting
- Performance status: ECOG 0
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - None
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: No known familial syndromes

Social History

- Smoking: Former smoker, quit 10 years ago (20 pack-year history)
- Alcohol use: Social drinking, 2-3 drinks per week
- Living situation/support: Lives with spouse, strong family support
- Occupation: Sales manager
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes

- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: NKDA (No Known Drug Allergies)

Physical Examination

- Vitals: BP 110/87, HR 71, RR 19, Temp 99.0°F, SpO2 96% on room air, Weight 128 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $4.3 \times 10^3/\mu\text{L}$, ANC: $6.9 \times 10^3/\mu\text{L}$
- Hgb: 14.3 g/dL, Platelets: $301 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.2 mg/dL, eGFR: 34 mL/min/1.73m²
- LFTs:
- AST: 86 U/L, ALT: 96 U/L
- Alkaline phosphatase: 281 U/L, Total bilirubin: 1.4 mg/dL
- CEA: 178.2 ng/mL (elevated)
- Molecular markers:
- RAS: NRAS Q61H
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 04, 2025):
- Primary sigmoid colon mass (6.2 cm), annular narrowing
- 6 hepatic metastases (largest 5.1 cm), enlarged lymph nodes (largest 1.6 cm), 3 brain metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (liver, lymph nodes, brain)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 16148888

Date of Visit: June 08, 2025

Oncologist: Dr. Sarah Mitchell, MD

Chief Complaint

- Change in bowel habits, Abdominal pain, Nausea

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 22, 2025
- Site of primary tumor: Upper rectum (rectum)
- Date of initial diagnosis and TNM stage: April 22, 2025; T3N1bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 16, 2025 - peritoneum, lymph nodes, liver metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Change in bowel habits, Abdominal pain, Nausea, Cramping, Diarrhea
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Sleep apnea
 - Asthma
- Surgical history:
 - Vasectomy (2007)
 - Circumcision (1995)
 - Carpal tunnel release (1995)

Family History

- Colorectal or other cancers in first-degree relatives: No known family history of cancer
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Never smoker
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Nurse
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency

- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Penicillin (rash), Shellfish (hives)

Physical Examination

- Vitals: BP 125/71, HR 98, RR 20, Temp 99.1°F, SpO2 98% on room air, Weight 128 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $11.8 \times 10^3/\mu\text{L}$, ANC: $6.3 \times 10^3/\mu\text{L}$
- Hgb: 8.7 g/dL, Platelets: $264 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.9 mg/dL, eGFR: 102 mL/min/1.73m²
- LFTs:
- AST: 48 U/L, ALT: 16 U/L
- Alkaline phosphatase: 143 U/L, Total bilirubin: 0.7 mg/dL
- CEA: 128.2 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 16, 2025):
- Primary upper rectum mass (3.6 cm), near-obstructing lesion
- 5 hepatic metastases (largest 5.6 cm), peritoneal disease, enlarged lymph nodes (largest 3.7 cm)
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (peritoneum, lymph nodes, liver)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 85414897

Date of Visit: May 23, 2025

Oncologist: Dr. Lisa Park, MD

Chief Complaint

- Vomiting, Cramping, Diarrhea

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 17, 2025
- Site of primary tumor: Descending colon (left colon)
- Date of initial diagnosis and TNM stage: April 17, 2025; T3N1aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 05, 2025 - bone metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Vomiting, Cramping, Diarrhea, Abdominal pain
- Performance status: ECOG 0
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Hypertension
 - Peripheral artery disease
 - Cataracts
 - Dementia (early stage)
- Surgical history:
 - Appendectomy (2014)

Family History

- Colorectal or other cancers in first-degree relatives: No known family history of cancer
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Former smoker, quit 5 years ago (15 pack-year history)
- Alcohol use: Social drinking, 2-3 drinks per week
- Living situation/support: Lives with spouse, strong family support
- Occupation: Small business owner
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency

- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Metoprolol 50mg twice daily
- Multivitamin daily
- Allergies: NKDA (No Known Drug Allergies)

Physical Examination

- Vitals: BP 119/81, HR 97, RR 16, Temp 98.5°F, SpO2 98% on room air, Weight 152 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $10.2 \times 10^3/\mu\text{L}$, ANC: $3.1 \times 10^3/\mu\text{L}$
- Hgb: 15.2 g/dL, Platelets: $420 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.9 mg/dL, eGFR: 76 mL/min/1.73m²
- LFTs:
- AST: 29 U/L, ALT: 27 U/L
- Alkaline phosphatase: 78 U/L, Total bilirubin: 1.1 mg/dL
- CEA: 61.6 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 05, 2025):
Primary descending colon mass (7.2 cm), polypoid mass
osseous metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (bone)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 47529475

Date of Visit: May 26, 2025

Oncologist: Dr. Michael Chen, MD

Chief Complaint

- Fatigue, Bloating, Vomiting

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 11, 2025
- Site of primary tumor: Descending colon (left colon)
- Date of initial diagnosis and TNM stage: April 11, 2025; T2N0M1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 07, 2025 - lung metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Fatigue, Bloating, Vomiting, Weight loss
- Performance status: ECOG 0
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - ADHD
 - GERD
- Surgical history:
 - Arthroscopy (1998)

Family History

- Colorectal or other cancers in first-degree relatives: Mother with breast cancer at age 72
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Never smoker
- Alcohol use: Former alcohol use, quit 5 years ago
- Living situation/support: Lives with spouse, strong family support
- Occupation: Nurse
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness

- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Multivitamin daily
- Allergies: Sulfa drugs (GI upset)

Physical Examination

- Vitals: BP 130/78, HR 91, RR 14, Temp 98.7°F, SpO2 96% on room air, Weight 137 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $3.5 \times 10^3/\mu\text{L}$, ANC: $6.3 \times 10^3/\mu\text{L}$
- Hgb: 15.3 g/dL, Platelets: $262 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.6 mg/dL, eGFR: 74 mL/min/1.73m²
- LFTs:
- AST: 29 U/L, ALT: 28 U/L
- Alkaline phosphatase: 62 U/L, Total bilirubin: 0.6 mg/dL
- CEA: 113.8 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: V600E
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 07, 2025):
- Primary descending colon mass (5.3 cm), annular narrowing
- 4 pulmonary nodules (largest 3.4 cm)
- Other findings: Degenerative spine changes
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lung)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 35162470

Date of Visit: May 28, 2025

Oncologist: Dr. Sarah Mitchell, MD

Chief Complaint

- Vomiting, Bloating, Early satiety

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 08, 2025
- Site of primary tumor: Transverse colon (right colon)
- Date of initial diagnosis and TNM stage: May 08, 2025; T2N2aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 15, 2025 - lung, bone, liver metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Vomiting, Bloating, Early satiety, Weight loss, Nausea
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - ADHD
 - Depression
- Surgical history:
 - Vasectomy (2001)
 - Cholecystectomy (2015)
 - Hernia repair (1995)
 - Circumcision (2012)

Family History

- Colorectal or other cancers in first-degree relatives: Paternal uncle with prostate cancer
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Never smoker
- Alcohol use: Former alcohol use, quit 5 years ago
- Living situation/support: Lives with spouse, strong family support
- Occupation: Nurse
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing

- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Shellfish (mild GI upset)

Physical Examination

- Vitals: BP 134/73, HR 83, RR 18, Temp 98.4°F, SpO2 99% on room air, Weight 188 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $9.2 \times 10^3/\mu\text{L}$, ANC: $8.3 \times 10^3/\mu\text{L}$
- Hgb: 9.1 g/dL, Platelets: $219 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.9 mg/dL, eGFR: 72 mL/min/1.73m²
- LFTs:
- AST: 92 U/L, ALT: 40 U/L
- Alkaline phosphatase: 79 U/L, Total bilirubin: 0.7 mg/dL
- CEA: 11.4 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: V600E
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 15, 2025):
- Primary transverse colon mass (3.8 cm), annular narrowing
- 7 hepatic metastases (largest 6.5 cm), 5 pulmonary nodules (largest 3.9 cm), osseous metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lung, bone, liver)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 57418600

Date of Visit: June 16, 2025

Oncologist: Dr. Jennifer Rodriguez, MD

Chief Complaint

- Abdominal pain, Rectal bleeding, Diarrhea

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 09, 2025
- Site of primary tumor: Mid rectum (rectum)
- Date of initial diagnosis and TNM stage: May 09, 2025; T4aN2bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 07, 2025 - liver metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Abdominal pain, Rectal bleeding, Diarrhea, Vomiting, Fatigue
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Psoriasis
 - Endometriosis
 - ADHD
 - Osteoarthritis
- Surgical history:
 - Arthroscopy (1991)

Family History

- Colorectal or other cancers in first-degree relatives: Father with colon cancer at age 68
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Former smoker, quit 5 years ago (15 pack-year history)
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Engineer
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency

- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Sulfa drugs (GI upset)

Physical Examination

- Vitals: BP 130/84, HR 72, RR 15, Temp 99.4°F, SpO2 99% on room air, Weight 186 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $12.1 \times 10^3/\mu\text{L}$, ANC: $2.4 \times 10^3/\mu\text{L}$
- Hgb: 13.7 g/dL, Platelets: $312 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.1 mg/dL, eGFR: 56 mL/min/1.73m²
- LFTs:
- AST: 26 U/L, ALT: 116 U/L
- Alkaline phosphatase: 254 U/L, Total bilirubin: 2.0 mg/dL
- CEA: 149.1 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSI-H

- Imaging:

- CT chest/abdomen/pelvis (June 07, 2025):
- Primary mid rectum mass (7.0 cm), polypoid mass
- 4 hepatic metastases (largest 4.5 cm)
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (liver)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 49271178

Date of Visit: June 14, 2025

Oncologist: Dr. Lisa Park, MD

Chief Complaint

- Change in bowel habits, Bloating, Diarrhea

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 05, 2025
- Site of primary tumor: Descending colon (left colon)
- Date of initial diagnosis and TNM stage: May 05, 2025; T2N1aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 03, 2025 - brain metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Change in bowel habits, Bloating, Diarrhea, Constipation, Abdominal pain, Weight loss
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Endometriosis
 - Surgical history:
 - Appendectomy (2023)

Family History

- Colorectal or other cancers in first-degree relatives: No known family history of cancer
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Current smoker (10 pack-years)
- Alcohol use: Social drinking, 2-3 drinks per week
- Living situation/support: Lives with spouse, strong family support
- Occupation: Office administrator
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes

- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Penicillin (rash), Shellfish (hives)

Physical Examination

- Vitals: BP 120/71, HR 95, RR 17, Temp 98.3°F, SpO2 100% on room air, Weight 149 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $6.1 \times 10^3/\mu\text{L}$, ANC: $7.6 \times 10^3/\mu\text{L}$
- Hgb: 13.5 g/dL, Platelets: $180 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.7 mg/dL, eGFR: 97 mL/min/1.73m²
- LFTs:
- AST: 38 U/L, ALT: 32 U/L
- Alkaline phosphatase: 91 U/L, Total bilirubin: 0.6 mg/dL
- CEA: 177.4 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSI-H

- Imaging:

- CT chest/abdomen/pelvis (June 03, 2025):
- Primary descending colon mass (4.7 cm), circumferential thickening
- 1 brain metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (brain)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 30086468

Date of Visit: May 31, 2025

Oncologist: Dr. Michael Chen, MD

Chief Complaint

- Diarrhea, Constipation, Cramping

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 04, 2025
- Site of primary tumor: Transverse colon (right colon)
- Date of initial diagnosis and TNM stage: April 04, 2025; T2N2bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 04, 2025 - peritoneum metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Diarrhea, Constipation, Cramping, Early satiety, Nausea, Fatigue
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Type 1 diabetes
 - Anxiety disorder
 - Celiac disease
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Father with colon cancer at age 68
- Known familial syndromes: No known familial syndromes

Social History

- Smoking: Never smoker
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Sales manager
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms

- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Shellfish (mild GI upset)

Physical Examination

- Vitals: BP 120/81, HR 88, RR 17, Temp 98.5°F, SpO2 98% on room air, Weight 127 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $4.9 \times 10^3/\mu\text{L}$, ANC: $2.3 \times 10^3/\mu\text{L}$
- Hgb: 14.9 g/dL, Platelets: $266 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.7 mg/dL, eGFR: 81 mL/min/1.73m²
- LFTs:
- AST: 32 U/L, ALT: 21 U/L
- Alkaline phosphatase: 82 U/L, Total bilirubin: 0.4 mg/dL
- CEA: 182.3 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: V600E
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 04, 2025):
- Primary transverse colon mass (2.6 cm), polypoid mass
- peritoneal disease
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (peritoneum)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 14728493

Date of Visit: May 26, 2025

Oncologist: Dr. Jennifer Rodriguez, MD

Chief Complaint

- Fatigue, Bloating, Nausea

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 28, 2025
- Site of primary tumor: Transverse colon (right colon)
- Date of initial diagnosis and TNM stage: April 28, 2025; T4aN1bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 16, 2025 - bone, liver, peritoneum metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Fatigue, Bloating, Nausea, Diarrhea, Constipation, Rectal bleeding
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Dementia (early stage)
 - Macular degeneration
 - Heart failure
 - Peripheral artery disease
 - Osteoporosis
 - Chronic kidney disease stage 2-3
- Surgical history:
 - Breast biopsy (1997)
 - Hernia repair (2001)
 - Skin lesion removal (2024)
 - Hysterectomy (2003)

Family History

- Colorectal or other cancers in first-degree relatives: Mother with breast cancer at age 72
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Never smoker
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired mechanic
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Multivitamin daily
- Allergies: Penicillin (rash)

Physical Examination

- Vitals: BP 121/84, HR 100, RR 15, Temp 98.6°F, SpO2 96% on room air, Weight 123 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $11.5 \times 10^3/\mu\text{L}$, ANC: $4.8 \times 10^3/\mu\text{L}$
- Hgb: 13.8 g/dL, Platelets: $284 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.9 mg/dL, eGFR: 83 mL/min/1.73m²
- LFTs:
- AST: 37 U/L, ALT: 13 U/L
- Alkaline phosphatase: 146 U/L, Total bilirubin: 1.0 mg/dL
- CEA: 175.8 ng/mL (elevated)
- Molecular markers:
- RAS: NRAS Q61H
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 16, 2025):
- Primary transverse colon mass (6.5 cm), polypoid mass
- 8 hepatic metastases (largest 6.4 cm), peritoneal disease, osseous metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (bone, liver, peritoneum)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 98503832

Date of Visit: May 18, 2025

Oncologist: Dr. Sarah Mitchell, MD

Chief Complaint

- Nausea, Diarrhea, Bloating

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 11, 2025
- Site of primary tumor: Lower rectum (rectum)
- Date of initial diagnosis and TNM stage: May 11, 2025; T4bN2bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 05, 2025 - brain metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Nausea, Diarrhea, Bloating, Weight loss
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Benign prostatic hyperplasia
 - Hypertension
 - Cataracts
 - Chronic kidney disease stage 2-3
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Former smoker, quit 10 years ago (20 pack-year history)
- Alcohol use: Rare alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Engineer
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency

- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Amlodipine 5mg daily
- Multivitamin daily
- Allergies: Shellfish (mild GI upset)

Physical Examination

- Vitals: BP 125/73, HR 85, RR 19, Temp 98.0°F, SpO2 98% on room air, Weight 193 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $5.2 \times 10^3/\mu\text{L}$, ANC: $5.9 \times 10^3/\mu\text{L}$
- Hgb: 15.7 g/dL, Platelets: $398 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.9 mg/dL, eGFR: 104 mL/min/1.73m²
- LFTs:
- AST: 19 U/L, ALT: 12 U/L
- Alkaline phosphatase: 125 U/L, Total bilirubin: 1.2 mg/dL
- CEA: 126.4 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G13D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 05, 2025):
- Primary lower rectum mass (6.3 cm), circumferential thickening
- 4 brain metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (brain)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 13264042

Date of Visit: May 22, 2025

Oncologist: Dr. Michael Chen, MD

Chief Complaint

- Nausea, Rectal bleeding, Early satiety

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 21, 2025
- Site of primary tumor: Sigmoid colon (left colon)
- Date of initial diagnosis and TNM stage: April 21, 2025; T4bN1bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 09, 2025 - brain, lymph nodes metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Nausea, Rectal bleeding, Early satiety, Weight loss, Abdominal pain
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Sleep apnea
 - Asthma
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Former smoker, quit 10 years ago (20 pack-year history)
- Alcohol use: Former alcohol use, quit 5 years ago
- Living situation/support: Lives with spouse, strong family support
- Occupation: Accountant
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness

- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Penicillin (rash)

Physical Examination

- Vitals: BP 127/80, HR 95, RR 16, Temp 98.4°F, SpO2 97% on room air, Weight 153 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $5.5 \times 10^3/\mu\text{L}$, ANC: $6.6 \times 10^3/\mu\text{L}$
- Hgb: 13.3 g/dL, Platelets: $300 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.8 mg/dL, eGFR: 76 mL/min/1.73m²
- LFTs:
- AST: 30 U/L, ALT: 26 U/L
- Alkaline phosphatase: 136 U/L, Total bilirubin: 0.2 mg/dL
- CEA: 151.9 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: V600E
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 09, 2025):
- Primary sigmoid colon mass (6.1 cm), polypoid mass
- enlarged lymph nodes (largest 3.1 cm), 3 brain metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (brain, lymph nodes)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 86504025

Date of Visit: May 18, 2025

Oncologist: Dr. Michael Chen, MD

Chief Complaint

- Nausea, Abdominal pain, Weight loss

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 07, 2025
- Site of primary tumor: Ascending colon (right colon)
- Date of initial diagnosis and TNM stage: May 07, 2025; T4aN0M1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 15, 2025 - lung, peritoneum metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Nausea, Abdominal pain, Weight loss, Early satiety, Fatigue, Cramping
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Migraine headaches
 - Depression
 - PCOS
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Paternal uncle with prostate cancer
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Former smoker, quit 10 years ago (20 pack-year history)
- Alcohol use: Rare alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Accountant
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms

- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Penicillin (rash), Shellfish (hives)

Physical Examination

- Vitals: BP 128/76, HR 80, RR 15, Temp 98.9°F, SpO2 99% on room air, Weight 163 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $14.1 \times 10^3/\mu\text{L}$, ANC: $3.9 \times 10^3/\mu\text{L}$
- Hgb: 12.1 g/dL, Platelets: $342 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.9 mg/dL, eGFR: 82 mL/min/1.73m²
- LFTs:
- AST: 75 U/L, ALT: 60 U/L
- Alkaline phosphatase: 272 U/L, Total bilirubin: 1.7 mg/dL
- CEA: 11.9 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G13D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 15, 2025):
- Primary ascending colon mass (6.0 cm), polypoid mass
- 1 pulmonary nodules (largest 1.0 cm), peritoneal disease
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lung, peritoneum)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 35879553

Date of Visit: May 26, 2025

Oncologist: Dr. Sarah Mitchell, MD

Chief Complaint

- Cramping, Early satiety, Bloating

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 16, 2025
- Site of primary tumor: Descending colon (left colon)
- Date of initial diagnosis and TNM stage: May 16, 2025; T3N0M1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 07, 2025 - bone metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Cramping, Early satiety, Bloating, Fatigue, Constipation
- Performance status: ECOG 0
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Inflammatory bowel disease
 - Anxiety disorder
 - Sleep apnea
 - Fibromyalgia
- Surgical history:
 - Testicular surgery (2006)
 - Circumcision (2005)

Family History

- Colorectal or other cancers in first-degree relatives: Paternal uncle with prostate cancer
- Known familial syndromes: No known familial syndromes

Social History

- Smoking: Never smoker
- Alcohol use: Former alcohol use, quit 5 years ago
- Living situation/support: Lives with spouse, strong family support
- Occupation: Engineer
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing

- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Penicillin (rash), Shellfish (hives)

Physical Examination

- Vitals: BP 120/71, HR 72, RR 20, Temp 98.1°F, SpO2 98% on room air, Weight 193 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $10.1 \times 10^3/\mu\text{L}$, ANC: $5.8 \times 10^3/\mu\text{L}$
- Hgb: 14.4 g/dL, Platelets: $250 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.7 mg/dL, eGFR: 107 mL/min/1.73m²
- LFTs:
- AST: 23 U/L, ALT: 30 U/L
- Alkaline phosphatase: 78 U/L, Total bilirubin: 0.7 mg/dL
- CEA: 117.6 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSI-H

- Imaging:

- CT chest/abdomen/pelvis (June 07, 2025):
- Primary descending colon mass (3.3 cm), circumferential thickening
- osseous metastases
- Other findings: Cholelithiasis, Pulmonary nodules (likely benign)
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (bone)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time

- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 86140084

Date of Visit: May 24, 2025

Oncologist: Dr. Sarah Mitchell, MD

Chief Complaint

- Abdominal pain, Vomiting, Cramping

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 12, 2025
- Site of primary tumor: Splenic flexure (left colon)
- Date of initial diagnosis and TNM stage: May 12, 2025; T4bN1aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 04, 2025 - lymph nodes, brain, bone metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Abdominal pain, Vomiting, Cramping, Early satiety
- Performance status: ECOG 0
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Asthma
 - Chronic fatigue syndrome
- Surgical history:
 - Tonsillectomy (2010)

Family History

- Colorectal or other cancers in first-degree relatives: Paternal uncle with prostate cancer
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Never smoker
- Alcohol use: Rare alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Small business owner
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness

- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Penicillin (rash), Shellfish (hives)

Physical Examination

- Vitals: BP 124/81, HR 83, RR 15, Temp 98.2°F, SpO2 100% on room air, Weight 173 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $5.5 \times 10^3/\mu\text{L}$, ANC: $2.5 \times 10^3/\mu\text{L}$
- Hgb: 13.4 g/dL, Platelets: $273 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.0 mg/dL, eGFR: 86 mL/min/1.73m²
- LFTs:
- AST: 19 U/L, ALT: 27 U/L
- Alkaline phosphatase: 95 U/L, Total bilirubin: 0.9 mg/dL
- CEA: 84.6 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: V600E
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 04, 2025):
- Primary splenic flexure mass (2.9 cm), near-obstructing lesion
- enlarged lymph nodes (largest 3.8 cm), osseous metastases, 2 brain metastases
- Other findings: Pulmonary nodules (likely benign)
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lymph nodes, brain, bone)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 70906771

Date of Visit: May 24, 2025

Oncologist: Dr. Sarah Mitchell, MD

Chief Complaint

- Early satiety, Bloating, Vomiting

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 10, 2025
- Site of primary tumor: Cecum (right colon)
- Date of initial diagnosis and TNM stage: May 10, 2025; T4bN1aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 13, 2025 - lymph nodes, bone metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Early satiety, Bloating, Vomiting, Change in bowel habits, Cramping
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Inflammatory bowel disease
 - Chronic fatigue syndrome
 - Endometriosis
- Surgical history:
 - Breast biopsy (2001)
 - Appendectomy (2016)
 - Hysterectomy (2021)

Family History

- Colorectal or other cancers in first-degree relatives: Paternal uncle with prostate cancer
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Never smoker
- Alcohol use: Rare alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Nurse
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing

- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Shellfish (mild GI upset)

Physical Examination

- Vitals: BP 121/77, HR 98, RR 15, Temp 98.5°F, SpO2 99% on room air, Weight 160 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $6.9 \times 10^3/\mu\text{L}$, ANC: $11.4 \times 10^3/\mu\text{L}$
- Hgb: 10.8 g/dL, Platelets: $258 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.7 mg/dL, eGFR: 110 mL/min/1.73m²
- LFTs:
- AST: 91 U/L, ALT: 94 U/L
- Alkaline phosphatase: 259 U/L, Total bilirubin: 1.9 mg/dL
- CEA: 138.3 ng/mL (elevated)
- Molecular markers:
- RAS: NRAS Q61H
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 13, 2025):
Primary cecum mass (4.4 cm), polypoid mass
enlarged lymph nodes (largest 1.9 cm), osseous metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lymph nodes, bone)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 18118308

Date of Visit: May 30, 2025

Oncologist: Dr. David Thompson, MD

Chief Complaint

- Diarrhea, Weight loss, Bloating

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 06, 2025
- Site of primary tumor: Splenic flexure (left colon)
- Date of initial diagnosis and TNM stage: May 06, 2025; T4bN2aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 03, 2025 - brain metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Diarrhea, Weight loss, Bloating, Fatigue
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - PCOS
 - Fibromyalgia
 - Anxiety disorder
 - Inflammatory bowel disease
- Surgical history:
 - Appendectomy (2014)

Family History

- Colorectal or other cancers in first-degree relatives: No known family history of cancer
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Former smoker, quit 10 years ago (20 pack-year history)
- Alcohol use: Former alcohol use, quit 5 years ago
- Living situation/support: Lives with spouse, strong family support
- Occupation: Sales manager
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency

- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Multivitamin daily
- Allergies: NKDA (No Known Drug Allergies)

Physical Examination

- Vitals: BP 111/71, HR 86, RR 15, Temp 98.6°F, SpO2 99% on room air, Weight 134 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $3.8 \times 10^3/\mu\text{L}$, ANC: $10.7 \times 10^3/\mu\text{L}$
- Hgb: 11.7 g/dL, Platelets: $208 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.7 mg/dL, eGFR: 87 mL/min/1.73m²
- LFTs:
- AST: 34 U/L, ALT: 62 U/L
- Alkaline phosphatase: 125 U/L, Total bilirubin: 1.5 mg/dL
- CEA: 160.1 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: V600E
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 03, 2025):
- Primary splenic flexure mass (7.8 cm), near-obstructing lesion
- 2 brain metastases
- Other findings: Pulmonary nodules (likely benign), Renal cysts, Atherosclerotic calcifications
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (brain)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 11533179

Date of Visit: June 15, 2025

Oncologist: Dr. Sarah Mitchell, MD

Chief Complaint

- Nausea, Abdominal pain, Bloating

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 19, 2025
- Site of primary tumor: Lower rectum (rectum)
- Date of initial diagnosis and TNM stage: April 19, 2025; T4bN2aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 16, 2025 - liver metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Nausea, Abdominal pain, Bloating, Fatigue, Early satiety, Cramping
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Thyroid disorder
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Father with colon cancer at age 68
- Known familial syndromes: No known familial syndromes

Social History

- Smoking: Never smoker
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired mechanic
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes

- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: NKDA (No Known Drug Allergies)

Physical Examination

- Vitals: BP 136/77, HR 83, RR 18, Temp 98.1°F, SpO2 96% on room air, Weight 148 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $8.1 \times 10^3/\mu\text{L}$, ANC: $4.0 \times 10^3/\mu\text{L}$
- Hgb: 14.1 g/dL, Platelets: $345 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.7 mg/dL, eGFR: 106 mL/min/1.73m²
- LFTs:
- AST: 57 U/L, ALT: 111 U/L
- Alkaline phosphatase: 236 U/L, Total bilirubin: 2.0 mg/dL
- CEA: 121.2 ng/mL (elevated)
- Molecular markers:
- RAS: NRAS Q61H
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 16, 2025):
- Primary lower rectum mass (5.4 cm), circumferential thickening
- 2 hepatic metastases (largest 4.6 cm)
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (liver)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 30447231

Date of Visit: May 31, 2025

Oncologist: Dr. Jennifer Rodriguez, MD

Chief Complaint

- Constipation, Early satiety, Vomiting

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 07, 2025
- Site of primary tumor: Ascending colon (right colon)
- Date of initial diagnosis and TNM stage: April 07, 2025; T3N1bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 12, 2025 - peritoneum, lymph nodes metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Constipation, Early satiety, Vomiting, Rectal bleeding, Diarrhea
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - PCOS
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Father with colon cancer at age 68
- Known familial syndromes: No known familial syndromes

Social History

- Smoking: Never smoker
- Alcohol use: Social drinking, 2-3 drinks per week
- Living situation/support: Lives with spouse, strong family support
- Occupation: Accountant
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes

- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Penicillin (rash)

Physical Examination

- Vitals: BP 116/89, HR 75, RR 20, Temp 98.6°F, SpO2 96% on room air, Weight 153 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $9.2 \times 10^3/\mu\text{L}$, ANC: $7.5 \times 10^3/\mu\text{L}$
- Hgb: 13.3 g/dL, Platelets: $387 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.9 mg/dL, eGFR: 88 mL/min/1.73m²
- LFTs:
- AST: 33 U/L, ALT: 10 U/L
- Alkaline phosphatase: 73 U/L, Total bilirubin: 1.1 mg/dL
- CEA: 156.3 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSI-H

- Imaging:

- CT chest/abdomen/pelvis (June 12, 2025):
Primary ascending colon mass (4.8 cm), near-obstructing lesion
peritoneal disease, enlarged lymph nodes (largest 2.4 cm)
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (peritoneum, lymph nodes)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 48286215

Date of Visit: May 31, 2025

Oncologist: Dr. David Thompson, MD

Chief Complaint

- Cramping, Bloating, Diarrhea

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 13, 2025
- Site of primary tumor: Lower rectum (rectum)
- Date of initial diagnosis and TNM stage: May 13, 2025; T4bN2bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 07, 2025 - brain metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Cramping, Bloating, Diarrhea, Constipation, Fatigue
- Performance status: ECOG 0
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - None
- Surgical history:
 - Cholecystectomy (2006)
 - Tonsillectomy (2024)

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Former smoker, quit 10 years ago (20 pack-year history)
- Alcohol use: Former alcohol use, quit 5 years ago
- Living situation/support: Lives with spouse, strong family support
- Occupation: Engineer
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness

- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Penicillin (rash)

Physical Examination

- Vitals: BP 132/89, HR 100, RR 18, Temp 98.1°F, SpO2 99% on room air, Weight 204 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $6.0 \times 10^3/\mu\text{L}$, ANC: $2.8 \times 10^3/\mu\text{L}$
- Hgb: 13.2 g/dL, Platelets: $216 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.2 mg/dL, eGFR: 84 mL/min/1.73m²
- LFTs:
- AST: 23 U/L, ALT: 14 U/L
- Alkaline phosphatase: 81 U/L, Total bilirubin: 0.2 mg/dL
- CEA: 77.0 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: V600E
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 07, 2025):
- Primary lower rectum mass (2.9 cm), circumferential thickening
- 4 brain metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (brain)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 57569409

Date of Visit: May 22, 2025

Oncologist: Dr. Sarah Mitchell, MD

Chief Complaint

- Bloating, Weight loss, Fatigue

History of Present Illness

- Date of initial colorectal cancer diagnosis: March 31, 2025
- Site of primary tumor: Ascending colon (right colon)
- Date of initial diagnosis and TNM stage: March 31, 2025; T3N1bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 09, 2025 - liver, lung metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Bloating, Weight loss, Fatigue
- Performance status: ECOG 0
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
- Migraine headaches
- Surgical history:
- Circumcision (2010)

Family History

- Colorectal or other cancers in first-degree relatives: No known family history of cancer
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Never smoker
- Alcohol use: Rare alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Engineer
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes

- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Penicillin (rash)

Physical Examination

- Vitals: BP 126/89, HR 76, RR 20, Temp 99.4°F, SpO2 100% on room air, Weight 185 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $5.3 \times 10^3/\mu\text{L}$, ANC: $11.1 \times 10^3/\mu\text{L}$
- Hgb: 10.5 g/dL, Platelets: $370 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.1 mg/dL, eGFR: 56 mL/min/1.73m²
- LFTs:
- AST: 54 U/L, ALT: 79 U/L
- Alkaline phosphatase: 97 U/L, Total bilirubin: 2.9 mg/dL
- CEA: 154.7 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: V600E
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 09, 2025):
- Primary ascending colon mass (4.2 cm), polypoid mass
- 5 hepatic metastases (largest 5.7 cm), 2 pulmonary nodules (largest 3.8 cm)
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (liver, lung)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 47601870

Date of Visit: May 28, 2025

Oncologist: Dr. Lisa Park, MD

Chief Complaint

- Weight loss, Diarrhea, Fatigue

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 27, 2025
- Site of primary tumor: Upper rectum (rectum)
- Date of initial diagnosis and TNM stage: April 27, 2025; T4bN1aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 05, 2025 - bone metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Weight loss, Diarrhea, Fatigue, Abdominal pain, Constipation, Bloating
- Performance status: ECOG 0
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
- Chronic fatigue syndrome
- PCOS
- Surgical history:
- Vasectomy (1994)
- Hernia repair (1991)
- Testicular surgery (2022)
- Arthroscopy (2012)

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: Lynch syndrome - negative

Social History

- Smoking: Never smoker
- Alcohol use: Social drinking, 2-3 drinks per week
- Living situation/support: Lives with spouse, strong family support
- Occupation: Office administrator
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing

- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Shellfish (mild GI upset)

Physical Examination

- Vitals: BP 121/72, HR 72, RR 19, Temp 98.5°F, SpO2 97% on room air, Weight 172 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $3.7 \times 10^3/\mu\text{L}$, ANC: $7.8 \times 10^3/\mu\text{L}$
- Hgb: 10.8 g/dL, Platelets: $166 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.9 mg/dL, eGFR: 113 mL/min/1.73m²
- LFTs:
- AST: 85 U/L, ALT: 69 U/L
- Alkaline phosphatase: 210 U/L, Total bilirubin: 0.7 mg/dL
- CEA: 188.5 ng/mL (elevated)
- Molecular markers:
- RAS: NRAS Q61H
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 05, 2025):
Primary upper rectum mass (2.9 cm), near-obstructing lesion
osseous metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (bone)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 15517509

Date of Visit: June 15, 2025

Oncologist: Dr. Jennifer Rodriguez, MD

Chief Complaint

- Early satiety, Abdominal pain, Nausea

History of Present Illness

- Date of initial colorectal cancer diagnosis: May 15, 2025
- Site of primary tumor: Cecum (right colon)
- Date of initial diagnosis and TNM stage: May 15, 2025; T2N1aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 09, 2025 - peritoneum metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Early satiety, Abdominal pain, Nausea
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Type 1 diabetes
 - Anxiety disorder
 - PCOS
- Surgical history:
 - Skin lesion removal (1998)

Family History

- Colorectal or other cancers in first-degree relatives: Paternal uncle with prostate cancer
- Known familial syndromes: No known familial syndromes

Social History

- Smoking: Current smoker (10 pack-years)
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Small business owner
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms

- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Shellfish (mild GI upset)

Physical Examination

- Vitals: BP 122/85, HR 71, RR 17, Temp 99.0°F, SpO2 98% on room air, Weight 132 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $4.3 \times 10^3/\mu\text{L}$, ANC: $4.2 \times 10^3/\mu\text{L}$
- Hgb: 14.9 g/dL, Platelets: $323 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.1 mg/dL, eGFR: 115 mL/min/1.73m²
- LFTs:
- AST: 22 U/L, ALT: 31 U/L
- Alkaline phosphatase: 60 U/L, Total bilirubin: 0.4 mg/dL
- CEA: 179.7 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 09, 2025):
- Primary cecum mass (4.5 cm), near-obstructing lesion
- peritoneal disease
- Other findings: Cholelithiasis, Atherosclerotic calcifications, Hiatal hernia
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (peritoneum)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 66569548

Date of Visit: June 04, 2025

Oncologist: Dr. David Thompson, MD

Chief Complaint

- Nausea, Abdominal pain, Rectal bleeding

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 28, 2025
- Site of primary tumor: Hepatic flexure (right colon)
- Date of initial diagnosis and TNM stage: April 28, 2025; T4aN2bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 07, 2025 - bone, brain metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Nausea, Abdominal pain, Rectal bleeding, Cramping
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - ADHD
 - Psoriasis
- Surgical history:
 - Tubal ligation (2009)
 - Cesarean section (1999)

Family History

- Colorectal or other cancers in first-degree relatives: Father with colon cancer at age 68
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Former smoker, quit 10 years ago (20 pack-year history)
- Alcohol use: Former alcohol use, quit 5 years ago
- Living situation/support: Lives with spouse, strong family support
- Occupation: Sales manager
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms

- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: Sulfa drugs (GI upset)

Physical Examination

- Vitals: BP 132/73, HR 87, RR 16, Temp 98.4°F, SpO2 99% on room air, Weight 196 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $10.9 \times 10^3/\mu\text{L}$, ANC: $10.5 \times 10^3/\mu\text{L}$
- Hgb: 14.9 g/dL, Platelets: $187 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.4 mg/dL, eGFR: 83 mL/min/1.73m²
- LFTs:
- AST: 44 U/L, ALT: 75 U/L
- Alkaline phosphatase: 191 U/L, Total bilirubin: 1.1 mg/dL
- CEA: 22.8 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G13D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 07, 2025):
- Primary hepatic flexure mass (7.3 cm), near-obstructing lesion
- osseous metastases, 3 brain metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (bone, brain)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 50330369

Date of Visit: May 29, 2025

Oncologist: Dr. Jennifer Rodriguez, MD

Chief Complaint

- Early satiety, Constipation, Diarrhea

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 05, 2025
- Site of primary tumor: Mid rectum (rectum)
- Date of initial diagnosis and TNM stage: April 05, 2025; T4aN1aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 11, 2025 - bone, liver metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Early satiety, Constipation, Diarrhea
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Inflammatory bowel disease
 - Type 2 diabetes
 - Anxiety disorder
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Paternal uncle with prostate cancer
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Former smoker, quit 5 years ago (15 pack-year history)
- Alcohol use: Rare alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Engineer
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms

- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Glipizide 5mg daily
- Allergies: NKDA (No Known Drug Allergies)

Physical Examination

- Vitals: BP 122/87, HR 80, RR 20, Temp 98.1°F, SpO2 100% on room air, Weight 197 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $8.4 \times 10^3/\mu\text{L}$, ANC: $7.7 \times 10^3/\mu\text{L}$
- Hgb: 12.5 g/dL, Platelets: $287 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.8 mg/dL, eGFR: 92 mL/min/1.73m²
- LFTs:
- AST: 36 U/L, ALT: 30 U/L
- Alkaline phosphatase: 60 U/L, Total bilirubin: 1.1 mg/dL
- CEA: 55.8 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 11, 2025):
- Primary mid rectum mass (5.8 cm), polypoid mass
- 3 hepatic metastases (largest 5.8 cm), osseous metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (bone, liver)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 63043709

Date of Visit: May 21, 2025

Oncologist: Dr. David Thompson, MD

Chief Complaint

- Rectal bleeding, Bloating, Vomiting

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 01, 2025
- Site of primary tumor: Cecum (right colon)
- Date of initial diagnosis and TNM stage: April 01, 2025; T3N2aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 15, 2025 - peritoneum, lung, bone metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Rectal bleeding, Bloating, Vomiting, Constipation, Weight loss
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Psoriasis
 - Celiac disease
 - Sleep apnea
- Surgical history:
 - None

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Never smoker
- Alcohol use: Rare alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired teacher
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms

- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- None
- Allergies: NKDA (No Known Drug Allergies)

Physical Examination

- Vitals: BP 138/71, HR 84, RR 20, Temp 99.5°F, SpO2 99% on room air, Weight 137 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $7.6 \times 10^3/\mu\text{L}$, ANC: $4.1 \times 10^3/\mu\text{L}$
- Hgb: 15.1 g/dL, Platelets: $343 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 0.8 mg/dL, eGFR: 98 mL/min/1.73m²
- LFTs:
- AST: 15 U/L, ALT: 14 U/L
- Alkaline phosphatase: 146 U/L, Total bilirubin: 1.0 mg/dL
- CEA: 123.3 ng/mL (elevated)
- Molecular markers:
- RAS: KRAS G12D
- BRAF: Wild-type
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 15, 2025):
- Primary cecum mass (4.7 cm), near-obstructing lesion
- 6 pulmonary nodules (largest 3.6 cm), peritoneal disease, osseous metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (peritoneum, lung, bone)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 82446651

Date of Visit: May 28, 2025

Oncologist: Dr. Jennifer Rodriguez, MD

Chief Complaint

- Rectal bleeding, Cramping, Fatigue

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 26, 2025
- Site of primary tumor: Splenic flexure (left colon)
- Date of initial diagnosis and TNM stage: April 26, 2025; T4aN2aM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 14, 2025 - liver, lymph nodes, bone metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Rectal bleeding, Cramping, Fatigue, Vomiting, Diarrhea
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Coronary artery disease
 - Heart failure
- Surgical history:
 - Pacemaker insertion (2024)

Family History

- Colorectal or other cancers in first-degree relatives: Mother with breast cancer at age 72
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Never smoker
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired teacher
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency
- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness

- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Multivitamin daily
- Allergies: Penicillin (rash)

Physical Examination

- Vitals: BP 110/89, HR 99, RR 18, Temp 99.3°F, SpO2 97% on room air, Weight 166 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $9.9 \times 10^3/\mu\text{L}$, ANC: $11.1 \times 10^3/\mu\text{L}$
- Hgb: 11.8 g/dL, Platelets: $282 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.2 mg/dL, eGFR: 57 mL/min/1.73m²
- LFTs:
- AST: 48 U/L, ALT: 101 U/L
- Alkaline phosphatase: 170 U/L, Total bilirubin: 1.9 mg/dL
- CEA: 47.2 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: V600E
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 14, 2025):
- Primary splenic flexure mass (4.7 cm), infiltrative mass
- 8 hepatic metastases (largest 5.3 cm), enlarged lymph nodes (largest 2.6 cm), osseous metastases
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (liver, lymph nodes, bone)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 84184554

Date of Visit: June 14, 2025

Oncologist: Dr. Sarah Mitchell, MD

Chief Complaint

- Fatigue, Vomiting, Diarrhea

History of Present Illness

- Date of initial colorectal cancer diagnosis: June 06, 2025
- Site of primary tumor: Ascending colon (right colon)
- Date of initial diagnosis and TNM stage: June 06, 2025; T3N0M1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 16, 2025 - lung, liver metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Fatigue, Vomiting, Diarrhea, Rectal bleeding, Cramping, Change in bowel habits
- Performance status: ECOG 2
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Hyperlipidemia
 - Depression
 - COPD
- Surgical history:
 - Vasectomy (2011)
 - Circumcision (2015)

Family History

- Colorectal or other cancers in first-degree relatives: Maternal grandmother with ovarian cancer
- Known familial syndromes: Genetic counseling pending

Social History

- Smoking: Current smoker (10 pack-years)
- Alcohol use: Denies alcohol use
- Living situation/support: Lives with spouse, strong family support
- Occupation: Retired teacher
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency

- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Simvastatin 40mg daily
- Multivitamin daily
- Allergies: Sulfa drugs (GI upset)

Physical Examination

- Vitals: BP 116/90, HR 73, RR 17, Temp 98.7°F, SpO2 100% on room air, Weight 178 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $3.9 \times 10^3/\mu\text{L}$, ANC: $4.0 \times 10^3/\mu\text{L}$
- Hgb: 12.8 g/dL, Platelets: $243 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.2 mg/dL, eGFR: 78 mL/min/1.73m²
- LFTs:
- AST: 34 U/L, ALT: 19 U/L
- Alkaline phosphatase: 142 U/L, Total bilirubin: 0.5 mg/dL
- CEA: 56.5 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: Wild-type
- MSI/MMR status: MSI-H

- Imaging:

- CT chest/abdomen/pelvis (June 16, 2025):
- Primary ascending colon mass (5.5 cm), circumferential thickening
- 2 hepatic metastases (largest 5.3 cm), 6 pulmonary nodules (largest 3.7 cm)
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (lung, liver)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms

MRN: 69472124

Date of Visit: June 08, 2025

Oncologist: Dr. Lisa Park, MD

Chief Complaint

- Vomiting, Constipation, Cramping

History of Present Illness

- Date of initial colorectal cancer diagnosis: April 14, 2025
- Site of primary tumor: Upper rectum (rectum)
- Date of initial diagnosis and TNM stage: April 14, 2025; T4aN1bM1 (Stage IV)
- Date and nature of metastatic disease diagnosis: June 09, 2025 - peritoneum metastases identified
- Summary of prior treatments: None - newly diagnosed, treatment-naïve
- Current symptoms: Vomiting, Constipation, Cramping
- Performance status: ECOG 1
- Recent complications or hospitalizations: None

Past Medical History

- Other malignancies: None
- Significant comorbidities:
 - Benign prostatic hyperplasia
 - Osteoarthritis
 - Atrial fibrillation
 - GERD
- Surgical history:
 - Tubal ligation (2012)

Family History

- Colorectal or other cancers in first-degree relatives: Father with colon cancer at age 68
- Known familial syndromes: No known familial syndromes

Social History

- Smoking: Former smoker, quit 5 years ago (15 pack-year history)
- Alcohol use: Former alcohol use, quit 5 years ago
- Living situation/support: Lives with spouse, strong family support
- Occupation: Sales manager
- Functional status/ADL: Independent in all activities of daily living

Review of Systems

- Constitutional: Fatigue, weight loss, no fever, chills, or night sweats
- GI: Abdominal pain, change in bowel habits, no hematemesis, early satiety
- Cardiovascular: No chest pain, palpitations, or shortness of breath
- Respiratory: No cough, dyspnea, or wheezing
- Genitourinary: No dysuria, hematuria, or urinary frequency

- Neurological: No headaches, dizziness, or focal neurological symptoms
- Musculoskeletal: No new joint pain or muscle weakness
- Integumentary: No rashes or skin changes
- Hematologic: No easy bruising or bleeding

Medications & Allergies

- Current medications (exclude current anti cancer therapies):
- Multivitamin daily
- Allergies: Sulfa drugs (GI upset)

Physical Examination

- Vitals: BP 116/80, HR 84, RR 15, Temp 98.1°F, SpO2 98% on room air, Weight 191 lbs
- General appearance: Alert and oriented, appears stated age, mild pallor
- Abdominal exam: Soft, mild tenderness, no masses palpable, no ascites, active bowel sounds
- Rectal exam: No masses palpable, guaiac positive stool
- Other relevant findings: No lymphadenopathy, no peripheral edema

Laboratory & Imaging Results

- Most recent CBC:
- WBC: $8.2 \times 10^3/\mu\text{L}$, ANC: $3.1 \times 10^3/\mu\text{L}$
- Hgb: 15.1 g/dL, Platelets: $202 \times 10^3/\mu\text{L}$
- Chemistry:
- Creatinine: 1.1 mg/dL, eGFR: 96 mL/min/1.73m²
- LFTs:
- AST: 25 U/L, ALT: 30 U/L
- Alkaline phosphatase: 92 U/L, Total bilirubin: 0.6 mg/dL
- CEA: 177.2 ng/mL (elevated)
- Molecular markers:
- RAS: Wild-type
- BRAF: V600E
- MSI/MMR status: MSS

- Imaging:

- CT chest/abdomen/pelvis (June 09, 2025):
- Primary upper rectum mass (3.6 cm), near-obstructing lesion
- peritoneal disease
- Other relevant tests: Colonoscopy with biopsy confirming moderately differentiated adenocarcinoma

Assessment

- Diagnosis: Metastatic colorectal cancer (peritoneum)
- Disease status: Newly diagnosed, treatment-naïve
- Treatment-related toxicities/adverse events: None at this time
- Symptom burden: Moderate - abdominal pain, constitutional symptoms